

Dexamethasone (systemic): Drug information - UpToDate

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For additional information see "Dexamethasone (systemic): Patient drug information" and "Dexamethasone (systemic): Pediatric drug information"

For abbreviations, symbols, and age group definitions show table

Brand Names: US

- Dexabliss [DSC];
- dexamETHasone Intensol;
- DoubleDex;
- Dxevo 11-Day [DSC];
- Hemaay;
- HiDex 6-Day;
- MAS Care-Pak;
- ReadySharp dexamETHasone [DSC];
- TaperDex 12-Day;
- TaperDex 6-Day;
- TaperDex 7-Day;
- TopiDex;
- ZCORT 7-Day [DSC]

Brand Names: Canada

- APO-Dexamethasone;
- Dexamethasone Omega Unidose;
- Dexamethasone-Omega;
- Olan-Dexamethasone;
- PMS-Dexamethasone;
- PMS-Dexamethasone Sod Phosphat [DSC]

Pharmacologic Category

- Anti-inflammatory Agent;
- Antiemetic;
- Corticosteroid, Systemic

Dosing: Adult

Dosage guidance:

Dosing: Individualize glucocorticoid dosing and use the minimum effective dose/duration. Evidence to support an optimal dose and duration is lacking for most indications; recommendations provided are general guidelines only.

Clinical considerations: In patients receiving chronic glucocorticoids (eg, ≥ 3 to 4 weeks) at supraphysiologic doses (>0.25 mg/day dexamethasone) **for indications other than adrenal insufficiency**, risk of hypothalamic-pituitary-adrenal axis suppression is increased. If glucocorticoid discontinuation is indicated, reduce dose gradually and monitor for signs of adrenal insufficiency. Higher doses may be needed during acute illness or surgery (Ref).

Usual dosage range: Oral, IV, IM: 4 to 20 mg/day given in a single daily dose or in 2 to 4 divided doses; *High dose:* 0.4 to 0.8 mg/kg/day (usually not to exceed 40 mg/day).

Acute mountain sickness/high-altitude cerebral edema

Acute mountain sickness/high-altitude cerebral edema (off-label use):

Prevention, moderate- to high-risk situations (alternative agent): Note: Use in addition to gradual ascent and start the day of ascent.

Oral: 2 mg every 6 hours **or** 4 mg every 12 hours; may be discontinued after staying at the same elevation for 2 to 4 days or if descent is initiated. In situations of rapid ascent to altitudes $>3,500$ meters (eg, rescue or military operations), 4 mg every 6 hours may be considered (Ref). Due to adverse effects, limit duration (Ref); some experts limit to ≤ 7 days (Ref).

Treatment:

Acute mountain sickness (moderate to severe): Note: Dexamethasone does not facilitate acclimatization; further ascent should be delayed until patient is asymptomatic off medication (Ref).

Oral, IM, IV: 4 mg every 6 hours, continue until 24 hours after symptoms resolve or descent completed (not longer than 7 days total) (Ref).

High-altitude cerebral edema: Oral, IM, IV: 8 mg as a single dose, followed by 4 mg every 6 hours until descent is complete and symptoms resolve (Ref).

Acute respiratory distress syndrome

Acute respiratory distress syndrome (off-label use): Note: While optimal timing of initiation of therapy is unclear, initiation of corticosteroid therapy beyond 2 weeks after onset of ARDS may be associated with harm (Ref).

IV: 20 mg once daily from days 1 to 5, then 10 mg once daily from days 6 to 10 (Ref).

Adrenal insufficiency, adrenal crisis

Adrenal insufficiency, adrenal crisis (alternative agent):

Note: Appropriate fluid resuscitation is also required.

Initial dosing: IV: 4 to 6 mg once daily; administer first dose immediately upon presentation (Ref).

Subsequent management: If clinical status has improved after 24 hours, may begin gradually tapering the dose. Once patient is stable, resume or begin chronic maintenance oral regimen (Ref). In patients with mineralocorticoid deficiency (eg, primary adrenal insufficiency), use in combination with fludrocortisone as soon as able to take oral medications (Ref).

Adrenal insufficiency, chronic

Adrenal insufficiency, chronic (alternative agent):

Note: For use in patients with severe symptoms of cortisol deficiency while using a short-acting glucocorticoid (eg, hydrocortisone) or who have difficulty adhering to multiple daily dosing regimens. Use in combination with fludrocortisone in patients with mineralocorticoid deficiency (eg, primary adrenal insufficiency) (Ref).

Maintenance dosing: Oral: Initial: 0.5 mg once daily (Ref). May adjust daily dose if needed based on signs and symptoms of under- or over-replacement; use the lowest effective dose. Usual dosage range: 0.25 to 0.75 mg/day (Ref).

Stress dosing: Note: For use in patients with chronic adrenal insufficiency (AI) and acute physiologic stressors (eg, illness, surgery) to prevent development of adrenal crisis. Patients without known chronic AI with recent or current chronic glucocorticoid use at supraphysiologic doses (eg, >0.5 mg/day dexamethasone) may have hypothalamic-pituitary-adrenal axis suppression and may require stress dosing; individualize treatment decisions in these patients (Ref). If a parenteral glucocorticoid is required (eg, unable to take oral medications, critical illness, labor/delivery, moderate/severe surgical stress), switch to hydrocortisone (Ref). Dosing provided is calculated based on hydrocortisone equivalency.

Acute physiologic stress/illness:

Febrile illness: Double the chronic maintenance dose for fever 38°C (100.4°F) to 39°C (102.2°F) **or** triple the chronic maintenance oral dose for fever >39°C (>102.2°F) or persistent nausea. Continue higher dose for 3 days, then return to baseline dose if fever has resolved. If fever has not resolved by day 4, further evaluation (with continued use of higher steroid doses if needed) is required (Ref).

Illness requiring hospitalization (eg, community acquired pneumonia): Oral: 2 to 3 mg once daily. For patients requiring a parenteral glucocorticoid, switch to hydrocortisone. Taper and return to baseline dose within 2 to 3 days following improvement of underlying condition (Ref). **Note:** For critical illness requiring intensive care, follow dosing recommendations for adrenal crisis (Ref).

Surgical stress:

Minor surgical stress (eg, hernia repair, procedures with local anesthetic): Oral: Give an additional 0.75 to 1 mg supplemental dose on the day of surgery (Ref).

Adrenal insufficiency due to classic congenital adrenal hyperplasia

Adrenal insufficiency due to classic congenital adrenal hyperplasia:

Note: For short-term use (alone or in combination with chronic glucocorticoid therapy) to facilitate shrinkage of testicular adrenal rest tumors. May also be used as an alternative chronic glucocorticoid when preferred agents (eg, hydrocortisone) are unavailable or cannot be taken. In patients with mineralocorticoid deficiency, use in combination with fludrocortisone. Avoid use during pregnancy (Ref).

Oral: 0.25 to 1 mg once daily at bedtime (Ref).

Stress dosing: Patients undergoing physiologic stress (eg, febrile illness, surgery, labor/delivery) require increased glucocorticoid doses to prevent adrenal crisis. Individualize treatment decisions; refer to dosing for “Adrenal insufficiency, chronic” for stress dosing instructions (Ref).

Antiemetic regimens: Chemotherapy-associated nausea and vomiting, prevention

Antiemetic regimens: Chemotherapy-associated nausea and vomiting, prevention (off-label use): Note: When checkpoint inhibitor therapy is administered in combination with emetogenic chemotherapy, there is no evidence to omit dexamethasone from the prophylactic antiemetic regimen (Ref).

Single-day IV chemotherapy regimens:

Highly emetogenic chemotherapy (>90% risk of emesis): Cisplatin and other highly emetogenic single agents:

Dexamethasone dose depends on specific neurokinin 1 (NK₁) receptor antagonist:

Day of chemotherapy: Administer prior to chemotherapy **and** in combination with an NK₁ receptor antagonist, a 5-HT₃ receptor antagonist, and olanzapine (Ref).

In combination with aprepitant, fosaprepitant, netupitant/palonosetron (NEPA), or fosnetupitant/palonosetron: **Oral, IV:** 12 mg (Ref).

In combination with rolapitant: **Oral, IV: 20 mg** (Ref).

If NK₁ receptor antagonist not used: **Oral, IV: 20 mg** (Ref).

Postchemotherapy days:

If aprepitant given: **Oral, IV: 8 mg** once daily on days 2 to 4 (Ref).

If fosaprepitant given: **Oral, IV: 8 mg** once on day 2, followed by 8 mg twice daily on days 3 and 4 (Ref).

If NEPA or fosnetupitant/palonosetron given: Prophylaxis with dexamethasone on subsequent days is not needed unless regimen contained cisplatin: **Oral, IV: 8 mg** once daily on days 2 to 4 (Ref).

If rolapitant given: **Oral, IV: 8 mg** twice daily on days 2 to 4 (Ref).

If NK₁ receptor antagonist not used: **Oral, IV: 8 mg** twice daily on days 2 to 4 (Ref).

Highly emetogenic chemotherapy (>90% risk of emesis): Breast cancer regimens that include an anthracycline combined with cyclophosphamide:

Dexamethasone dose depends on specific NK₁ receptor antagonist:

Day of chemotherapy: Administer prior to chemotherapy **and** in combination with an NK₁ receptor antagonist, a 5-HT₃ receptor antagonist, and olanzapine (Ref).

In combination with aprepitant, fosaprepitant, NEPA, fosnetupitant/palonosetron: **Oral, IV: 12 mg** (Ref).

In combination with rolapitant: **Oral, IV: 20 mg** (Ref).

If NK₁ receptor antagonist not used: **Oral, IV: 20 mg** (Ref).

Postchemotherapy days: Dexamethasone use is not recommended (an alternative agent or agents is/are recommended) (Ref).

Moderately emetogenic chemotherapy (30% to 90% risk of emesis): Carboplatin-based regimens:

Dexamethasone dose depends on specific NK₁ receptor antagonist (Ref):

Day of chemotherapy: Administer prior to chemotherapy **and** in combination with an NK₁ receptor antagonist and a 5-HT₃ receptor antagonist (Ref).

In combination with aprepitant, fosaprepitant, NEPA, or fosnetupitant/palonosetron: **Oral, IV: 12 mg** (Ref).

In combination with rolapitant: **Oral, IV: 20 mg** (Ref).

Postchemotherapy days: Prophylaxis is not necessary on subsequent days (Ref).

Moderately emetogenic chemotherapy (30% to 90% risk of emesis): Non-carboplatin-based regimens:

Day of chemotherapy: Administer prior to chemotherapy **and** in combination with a 5-HT₃ receptor antagonist: **Oral, IV: 8 mg** (Ref).

Postchemotherapy days: Note: Consider single-agent dexamethasone on days 2 and 3 and only for regimens containing agents with known potential to induce delayed emesis (eg, oxaliplatin, cyclophosphamide, doxorubicin) (Ref).

Oral, IV: 8 mg on days 2 and 3 (Ref).

Low emetogenic risk (10% to 30% risk of emesis): Oral, IV: 4 to 8 mg administered as a single agent in a single dose prior to chemotherapy; prophylaxis is not necessary on subsequent days (Ref).

High-dose conditioning chemotherapy with hematopoietic cell or bone marrow transplant:

Day(s) of conditioning chemotherapy: Administer prior to conditioning chemotherapy **and** in combination with a neurokinin 1 (NK₁) receptor antagonist, a 5-HT₃ receptor antagonist, with or without olanzapine (Ref).

Oral, IV: 8 to 20 mg on each day of chemotherapy (Ref).

Antiemetic regimens: Postoperative nausea and vomiting, prevention

Antiemetic regimens: Postoperative nausea and vomiting, prevention (off-label use):

Note: May be used alone or in combination with one or more other prophylactic interventions depending on risk factors (Ref).

IV: 4 to 10 mg once, before or after induction of anesthesia. Dose depends on risk and type of surgery (Ref).

Antiemetic regimens: Radiation therapy-associated nausea and vomiting, prevention

Antiemetic regimens: Radiation therapy-associated nausea and vomiting, prevention (off-label use):

High emetogenic risk radiation therapy (total body irradiation):

Radiation day(s): Oral, IV: 4 mg once daily prior to each fraction of radiation; give in combination with a 5-HT₃ receptor antagonist (Ref).

Postradiation days: Oral, IV: The appropriate duration of therapy following radiotherapy days is not well defined; American Society of Clinical Oncology (ASCO) guidelines recommend continuing dexamethasone 4 mg once on the day after each day of radiation if radiation is not planned for that day (Ref).

Moderate emetogenic risk radiation therapy (upper abdomen, craniospinal irradiation):

Radiation day(s): Oral, IV: 4 mg once daily prior to each of the first 5 fractions of radiation; give in combination with a 5-HT₃ receptor antagonist (Ref).

Total body irradiation with hematopoietic cell or bone marrow transplant:

Day(s) of total body irradiation: Oral, IV: 4 mg prior to each radiation session (Ref); give in combination with a 5-HT₃ receptor antagonist (Ref).

Postradiation day(s): Oral, IV: ASCO guidelines recommend continuing dexamethasone 4 mg once on the day after each day of radiation if radiation is not planned for that day (Ref).

Asthma, acute exacerbation

Asthma, acute exacerbation (alternative agent) (off-label use): Note: Alternative to a longer course of other corticosteroids in mild to moderate exacerbations or in patients who do not respond promptly and completely to short-acting beta-agonists; administer within 1 hour of presentation to emergency department (Ref).

Oral: 12 to 16 mg daily for 1 to 2 days only (Ref); longer treatment at this dose may be associated with metabolic adverse effects (Ref).

Cancer-related cachexia

Cancer-related cachexia (alternative agent) (off-label use): Oral: 3 to 4 mg once daily; short-term therapy (weeks) is recommended, although duration of treatment depends on treatment goals and risk/benefit assessment (Ref) **or** 3 to 6 mg/day for up to 4 weeks (Ref).

Cerebral edema associated with brain tumor

Cerebral (vasogenic) edema associated with brain tumor:

Impending herniation (eg, decreased responsiveness, new 3rd nerve pals y)

Initial: **IV:** 10 mg once followed by maintenance dosing (Ref).

Maintenance: **IV, Oral:** 16 to 40 mg/day in 2 to 4 divided doses (Ref).

Moderate to severe symptoms (eg, severe headache, nausea and vomiting, seizures, significant focal neurologic deficits)

Initial: **IV:** 10 mg once followed by maintenance dosing (Ref).

Maintenance: **IV, Oral:** 8 to 16 mg/day in 1 to 4 divided doses (Ref). **Note:** Consider taper after 7 days of therapy; taper slowly over several weeks (Ref).

Mild symptoms: IV, Oral: 4 to 8 mg/day in 1 to 4 divided doses (Ref). Some experts recommend lower doses of 2 to 4 mg/day in 1 to 2 divided doses to reduce adverse effects (Ref). **Note:** Consider taper after 7 days of therapy; taper slowly over several weeks (Ref).

COVID-19, hospitalized patients

COVID-19, hospitalized patients (off-label use):

Note: Dexamethasone is recommended for treatment of COVID-19 in hospitalized patients requiring supplemental oxygen or ventilatory support (Ref). An equivalent dose of an alternative glucocorticoid may be substituted if dexamethasone is unavailable (Ref).

IV, Oral: 6 mg once daily for up to 10 days (or until discharge if sooner) as part of an appropriate combination regimen (Ref).

Cushing syndrome, diagnosis

Cushing syndrome, diagnosis: Note: Interpretation requires evaluation of one or more of the following: serum cortisol concentration, serum dexamethasone concentration, urinary cortisol excretion, or 17-hydroxycorticosteroid excretion; consultation with a clinical endocrinologist is recommended (Ref).

Initial testing:

Overnight 1 mg dexamethasone suppression test: **Oral:** 1 mg given once between 11 PM and 12 AM (Ref).

Longer low-dose dexamethasone suppression test (2 mg/day for 48 hours): **Note:** May be preferred in patients with depression, anxiety, obsessive-compulsive disorder, morbid obesity, alcoholism, or diabetes mellitus (Ref).

Oral: 0.5 mg every 6 hours for 48 hours for a total of 8 doses; start time varies (eg, 9 AM or 12 PM) (Ref).

Fetal lung maturation, acceleration of

Fetal lung maturation, acceleration of (maternal administration) (off-label use): Note: Generally, for patients between 24 and 34 weeks of gestation, including those with ruptured membranes or multiple gestations, who are at risk of delivering within 7 days. A single course may be appropriate in some pregnant patients beginning at 22 weeks' gestation or late preterm (between 34 0/7 weeks' and 36 6/7 weeks' gestation) who are at risk of delivering within 7 days.

IM: 6 mg every 12 hours for a total of 4 doses. May repeat course in select patients (eg, patients with pregnancies up to 34 weeks' gestation at risk for delivery within 7 days and >14 days have elapsed since initial course of antenatal corticosteroids) (Ref).

Immune thrombocytopenia

Immune thrombocytopenia (initial therapy): Note: Goal of therapy is to provide a safe platelet count to prevent clinically important bleeding rather than normalization of the platelet count (Ref).

Oral, IV: 40 mg once daily for 4 days and then stop (no taper); may be repeated up to 3 times if inadequate response (Ref). For severe bleeding with thrombocytopenia, give in combination with other therapies (Ref).

Iodinated contrast media allergic-like reaction, prevention

Iodinated contrast media allergic-like reaction, prevention (alternative agent): Note: Generally for patients with a prior allergic-like or unknown-type iodinated contrast reaction who will be

receiving another iodinated contrast agent. Nonurgent premedication with an oral corticosteroid (eg, prednisone) is generally preferred when contrast administration is scheduled to begin in ≥ 12 hours; however, consider an urgent (accelerated) regimen with an IV corticosteroid for those requiring contrast in < 12 hours. Efficacy of premedication regimens starting < 4 to 5 hours before the use of contrast has not been demonstrated (Ref).

Urgent (accelerated) regimen: IV: 7.5 mg every 4 hours until contrast medium administration in combination with diphenhydramine (Ref).

Meningitis, prevention of neurologic complications

Meningitis (bacterial), prevention of neurologic complications (off-label use): Note: Administer first dose of dexamethasone at the same time as or within 1 hour of administration of the first dose of antibacterials (Ref); some experts still administer dexamethasone if 12 hours have passed since the first antibacterial dose was given (Ref). In patients with pneumococcal meningitis who receive dexamethasone, some experts recommend adding rifampin to the standard initial antibacterial regimen or adding rifampin if susceptibility tests, once available, show intermediate susceptibility ($\text{MIC} \geq 2$ mcg/mL) to ceftriaxone and cefotaxime (Ref).

Resource-abundant settings (suspected or confirmed pneumococcal meningitis): IV: 0.15 mg/kg/dose **or** 10 mg every 6 hours for 4 days (Ref).

Resource-limited regions (strongly suspected or confirmed bacterial meningitis): IV: 0.4 mg/kg/dose every 12 hours for 4 days; not recommended in regions with high rates of HIV infection and/or malnutrition or in cases of delayed clinical presentation (Ref).

Migraine, recurrence prevention

Migraine, recurrence prevention (off-label use): IM, IV: 4 to 24 mg once in combination with standard migraine abortive therapy (Ref). Some experts recommend a 4 mg dose due to evidence that higher doses are not more effective (Ref).

Multiple myeloma

Multiple myeloma : Note: Multiple dexamethasone-containing regimens are available. Refer to literature/guidelines for additional details. For many regimens, dexamethasone is continued until disease progression or unacceptable toxicity. Dexamethasone total weekly dose may be split over 2 days when combination therapies are administered on successive days (refer to protocol) (Ref).

Frail patients (eg, > 75 years of age, $\text{BMI} < 18.5 \text{ kg/m}^2$, poorly controlled diabetes, corticosteroid intolerance) : When administered weekly, dexamethasone is usually reduced to 20 mg once weekly for frail patients (Ref). May consider lower initial dexamethasone doses (8 to 20 mg once weekly) in patients > 75 years of age or those with comorbidities, with subsequent titration based on response/tolerance (Ref).

*Combination regimens that do **not** include a monoclonal antibody:*

Oral:

40 mg once weekly on days 1, 8, 15, and 22 every 28 days in combination with lenalidomide (Ref), pomalidomide (Ref), ixazomib and lenalidomide (Ref), ixazomib and lenalidomide for 18 cycles (Ref), carfilzomib and lenalidomide (Ref), or bortezomib and lenalidomide (Ref) **or** 40 mg once weekly on days 1, 8, 15, and 22 every 28 days in cycles 1 to 9, and then 40 mg once weekly on days 1, 8, and 15 every 28 days beginning at cycle 10 (in combination with carfilzomib) (Ref).

or

20 mg on days 1, 8, 15, and 22 every 28 days (in combination with lenalidomide) for 9 cycles, followed by lenalidomide maintenance (Ref) **or** 20 mg on days 1, 2, 4, 5, 8, 9, 11, and 12 every 21 days (in combination with bortezomib and lenalidomide) for 8 cycles (induction), followed by 40 mg on days 1, 8, 15, and 22 every 28 days (in combination with lenalidomide) for maintenance (Ref) **or** 20 mg on days 1, 2, 8, 9, 15, 16, 22, and 23 every 28 days (in combination with carfilzomib) (Ref) **or** 20 mg on days 1 and 3 of each week (in combination with selinexor) (Ref) **or** 20 mg on days 1, 2, 8, 9, 15, 16, 22, 23, 29, and 30 every 35 days (in combination with selinexor and bortezomib) (Ref).

or

40 mg once daily on days 1 to 4 and 9 to 12 every 28 days in combination with bortezomib and lenalidomide for 6 cycles followed by transplant (Ref) **or** 40 mg once daily on days 1 to 4, 9 to 12, and 17 to 20 every 28 days in combination with bortezomib and doxorubicin for 3 cycles as induction (Ref). **Note:** Some experts reserve this dosing (for 1 cycle, followed by 40 mg once weekly thereafter) for patients with an aggressive disease presentation or acute renal failure from light chain cast nephropathy (Ref).

Combination regimens that include a monoclonal antibody/antibody-drug conjugate:

IV, Oral:

BVd regimen: 20 mg once daily on days 1, 2, 4, 5, 8, 9, 11, and 12 of a 21-day treatment cycle in combination with belantamab, mafodotin, and bortezomib for 8 cycles (Ref).

DVRd regimen: 20 mg once daily on days 1, 2, 8, 9, 15, and 16 of a 21-day treatment cycle in combination with daratumumab, lenalidomide, and bortezomib (Ref) **or** 40 mg once daily on days 1 to 4 and 9 to 12 of a 28-day treatment cycle in combination with daratumumab/hyaluronidase, lenalidomide, and bortezomib (Ref) **or** 20 mg once daily on days 1, 2, 4, 5, 8, 9, 11, and 12 of a 21-day treatment cycle (days 1, 4, 8, and 11 in patients >75 years of age or BMI <18.5 kg/m²) in combination with daratumumab/hyaluronidase, lenalidomide, and bortezomib (Ref).

DVTd regimen: 40 mg once daily on days 1, 2, 8, 9, 15, 16, 22, and 23 of a 28-day treatment cycle for 2 cycles (induction cycles 1 and 2), followed by 40 mg once daily on days 1 and 2 and 20 mg once daily on days 8, 9, 15, and 16 of a 28-day treatment cycle for 2 cycles (induction cycles 3 and 4), followed by 20 mg once daily on days 1, 2, 8, 9, 15, and 16 of a 28-day treatment cycle for 2 cycles (consolidation following autologous hematopoietic cell transplant); in combination with bortezomib, daratumumab, and thalidomide (Ref).

DKd regimen: 40 mg once weekly **or** 20 mg once weekly (in patients >75 years of age); in combination with carfilzomib and daratumumab (Ref). **Note:** In some studies, the dexamethasone dose is split over 2 days (20 mg before daratumumab and 20 mg the day after daratumumab infusion) (Ref).

DRd regimen: 40 mg once weekly **or** 20 mg once weekly (in patients >75 years of age or BMI <18.5 kg/m²); in combination with daratumumab **or** daratumumab/hyaluronidase and lenalidomide (Ref). **Note:** In some studies, the dexamethasone dose is split over 2 days (20 mg before daratumumab and 20 mg the day after daratumumab infusion) (Ref).

DPd regimen: 40 mg once weekly **or** 20 mg once weekly (in patients >75 years of age); in combination with daratumumab **or** daratumumab/hyaluronidase and pomalidomide (Ref).

DVd regimen: 20 mg once daily on days 1, 2, 4, 5, 8, 9, 11, and 12 of a 21-day treatment cycle **or** 20 mg once weekly (in patients >75 years of age, BMI <18.5 kg/m², or with previous unacceptable adverse effects with previous glucocorticoids); in combination with bortezomib and daratumumab (Ref).

EPd regimen: 40 mg once weekly **or** 20 mg once weekly (in patients >75 years of age), except on days elotuzumab is administered (administer dexamethasone 28 mg orally [8 mg orally in patients >75 years of age] plus 8 mg IV prior to elotuzumab); in combination with elotuzumab and pomalidomide (Ref).

ERd regimen: 40 mg once weekly, except on days elotuzumab is administered (administer dexamethasone 28 mg orally plus 8 mg IV prior to elotuzumab); in combination with elotuzumab and lenalidomide (Ref).

IsaVRd regimen: 20 mg once daily on days 1, 2, 4, 5, 8, 9, 11, 12, 15, 22, 23, 25, 26, 29, 30, 32, and 33 (days 1, 4, 8, 11, 15, 22, 25, 29, and 32 in patients ≥75 years of age) in combination with bortezomib, isatuximab, and lenalidomide for 3 to 4 cycles, followed by 20 mg once weekly in combination with isatuximab and lenalidomide (Ref).

IsaKd regimen: 20 mg once daily on days 1, 2, 8, 9, 15, 16, 22, and 23 of a 28-day treatment cycle in combination with carfilzomib and isatuximab (Ref).

IsaPd regimen: 40 mg once weekly **or** 20 mg once weekly (in patients ≥ 75 years of age), in combination with isatuximab and pomalidomide (Ref).

Neoplastic epidural spinal cord compression, symptomatic

Neoplastic epidural spinal cord compression, symptomatic: Note: As an adjunct to definitive treatment (radiotherapy or surgery), particularly in patients with neurologic deficits (Ref).

IV (initial dose): 10 or 16 mg followed by oral dosing (Ref).

Oral (after IV dose): 16 mg/day (usually given in 2 to 4 divided doses). Once definitive treatment is underway, taper gradually over 1 to 2 weeks until discontinuation (Ref).

Tuberculosis, CNS

Tuberculosis, CNS: Note: In general, glucocorticoids are indicated for patients with established or suspected tuberculous meningitis, regardless of HIV status (Ref).

IV: Initial dose: 0.3 to 0.4 mg/kg/day for 2 weeks, then 0.2 mg/kg/day for week 3, then 0.1 mg/kg/day for week 4, followed by **oral** therapy (Ref).

Oral: Starting week 5 of treatment: 4 mg/day, then taper by 1 mg of the daily dose each week for 4 weeks; total combined IV/oral therapy duration: ~ 8 weeks (Ref); for patients with HIV, continue for 12 weeks (Ref).

Waldenström macroglobulinemia

Waldenström macroglobulinemia:

DRC regimen: **IV:** 20 mg on day 1 of a 21-day treatment cycle (in combination with rituximab and cyclophosphamide) for 6 cycles (Ref).

BDR regimen: **IV:** 40 mg on days 1, 4, 8, and 11 of a 21-day treatment cycle (in combination with bortezomib and rituximab) for 4 cycles, followed by a 12-week rest, then an additional 4 cycles (as an 84-day treatment cycle) for a total of 8 cycles (Ref) **or** 40 mg on days 1, 8, 15, and 22 of a 35-day treatment cycle during cycles 2 and 5 (in combination with bortezomib and rituximab) for a total of 5 cycles of therapy, with a total of 8 dexamethasone doses (Ref).

CaRD regimen: **IV:** 20 mg on days 1, 2, 8, and 9 of a 21-day treatment cycle (in combination with carfilzomib and rituximab) for 6 induction cycles, followed 8 weeks later by maintenance therapy of 20 mg on days 1 and 2 every 8 weeks (in combination with carfilzomib and rituximab) for 8 cycles (Ref).

Dosage adjustment for concomitant therapy: Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Dosing: Kidney Impairment: Adult

The renal dosing recommendations are based upon the best available evidence and clinical expertise. Senior Editorial Team: Bruce Mueller, PharmD, FCCP, FASN, FNKF; Jason A. Roberts, PhD, BPharm (Hons), B App Sc, FSHP, FISAC; Michael Heung, MD, MS.

Note: The pharmacokinetics and pharmacodynamics of corticosteroids in kidney impairment are not well understood (Ref). Dexamethasone half-life is decreased in patients with severe kidney impairment (Ref), potentially due to decreased protein binding (Ref); however, the clinical implications of these findings are unclear.

Altered kidney function: Oral, parenteral: No dosage adjustment necessary for any degree of kidney dysfunction (Ref).

Hemodialysis, intermittent (thrice weekly): Oral, parenteral: No supplemental dose or dosage adjustment necessary (Ref).

Peritoneal dialysis: Oral, parenteral: No dosage adjustment necessary (Ref).

CRRT: Oral, parenteral: No dosage adjustment necessary (Ref).

PIRRT (eg, sustained, low-efficiency diafiltration): Oral, parenteral: No dosage adjustment necessary (Ref).

Dosing: Liver Impairment: Adult

There are no dosage adjustments provided in the manufacturer's labeling.

Dosing: Obesity: Adult

The recommendations for dosing in patients with obesity are based upon the best available evidence and clinical expertise. Senior Editorial Team: Jeffrey F. Barletta, PharmD, FCCM; Manjunath P. Pai, PharmD, FCP; Jason A. Roberts, PhD, BPharm (Hons), B App Sc, FSHP, FISAC.

Class 1, 2 and 3 obesity (BMI ≥ 30 kg/m²):

Oral, IV, IM:

Non-weight-based dosing: No dosage adjustment necessary (Ref). Refer to adult dosing for indication-specific doses.

Weight-based dosing: Use **ideal body weight** to avoid overdosing and subsequent toxicity (Ref). Refer to adult dosing for indication-specific doses.

Rationale for recommendations: Corticosteroids are lipophilic compounds; however, the reported pharmacokinetic variability due to obesity is limited and inconsistent. Two small studies evaluated pharmacokinetic changes with oral dexamethasone in patients with obesity. One study reported lower AUC in patients with obesity compared to patients with normal weight (Ref), while another study reported a positive correlation between actual body weight and AUC (Ref). Weight-based dosing using actual body weight could lead to supratherapeutic levels (Ref).

Dosing: Older Adult

Refer to adult dosing. Use cautiously in older adults at the lowest possible dose.

Dosing: Pediatric

(For additional information see "Dexamethasone (systemic): Pediatric drug information")

Acute mountain sickness/high-altitude cerebral edema, treatment

Acute mountain sickness (AMS) (moderate)/high-altitude cerebral edema (HACE), treatment: Limited data available: **Note:** Dexamethasone does not facilitate acclimatization; further ascent should be delayed until patient is asymptomatic off medication (Ref).

Infants, Children, and Adolescents: Oral, IM, IV: 0.15 mg/kg/dose every 6 hours; maximum dose: 4 mg/dose (Ref).

Airway edema or extubation

Airway edema or extubation: Limited data available: Infants, Children, and Adolescents: Oral, IM, IV: 0.5 mg/kg/dose (maximum dose: 10 mg/dose) administered 6 to 12 hours prior to extubation then every 6 hours for 6 doses (total dexamethasone dose: 3 mg/kg) (Ref).

Anti-inflammatory

Anti-inflammatory: Infants, Children, and Adolescents: Oral, IM, IV: Initial dose range: 0.02 to 0.3 mg/kg/day or 0.6 to 9 mg/m²/day in divided doses every 6 to 12 hours; dose depends upon condition being treated and response of patient; dosage for infants and children should be based on disease severity and patient response; usual adult initial daily dose range: 0.75 to 9 mg/day.

Asthma exacerbation

Asthma exacerbation: Limited data available:

Infants, Children, and Adolescents: Oral, IM, IV: 0.3 to 0.6 mg/kg once daily for 1 to 2 days; maximum dose: 16 mg/dose (Ref). **Note:** If relapse occurs after 2 days of dexamethasone, change to prednisolone or prednisone (Ref).

Bacterial meningitis

Bacterial meningitis: Limited data available:

Note: Benefit is uncertain in young infants (eg, less than 3 months of age); weigh risks and benefits. Use in disease caused by pathogens other than *Haemophilus influenzae* or *Streptococcus pneumoniae* is typically not recommended; consider discontinuation if another pathogen is identified (Ref).

Infants, Children, and Adolescents:

IV: 0.15 mg/kg/dose every 6 hours for the first 2 to 4 days of antibiotic treatment; usual maximum dose: 10 mg/dose (Ref). Initiate dexamethasone preferably 10 to 20 minutes before or with the first dose of antibiotic (Ref); if not possible, may consider administration within 12 hours of first antibiotic dose (Ref).

Cerebral edema

Cerebral edema: Limited data available: **Note:** Dose, route, and duration may vary due to underlying cause of edema; tapering may be required. Infants, Children, and Adolescents: Oral, IM, IV: Loading dose: 1 to 2 mg/kg/dose as a single dose; maintenance: 1 to 2 mg/kg/**day** in divided doses every 4 to 6 hours; maximum daily dose: 16 mg/**day** (Ref).

Chemotherapy-induced nausea and vomiting, prevention

Chemotherapy-induced nausea and vomiting, prevention: Reported regimens variable; optimal dose not established (Ref). Refer to individual protocols and emetogenic potential:

Infants, Children, and Adolescents:

POGO recommendations (Ref): **Note:** Reduce dose by 50% if administered concomitantly with aprepitant:

Highly/severely emetogenic chemotherapy: Oral, IV: 6 mg/**m**²/dose every 6 hours.

Moderately emetogenic chemotherapy: Oral, IV:

BSA ≤ 0.6 m²: 2 mg every 12 hours.

BSA > 0.6 m²: 4 mg every 12 hours.

Alternate dosing: *Highly/severely emetogenic chemotherapy:* IV: Usual: 10 mg/**m**²/dose once daily on days of chemotherapy; some patients may require every 12-hour dosing; usual range: 8 to 14 mg/**m**²/dose (Ref).

Congenital adrenal hyperplasia, maintenance

Congenital adrenal hyperplasia, maintenance: Adolescents (fully grown): Oral: 0.25 to 0.5 mg once daily; use of a liquid dosage form may be preferable to allow for better dose titration (Ref).

Note: For younger patients who are still growing, hydrocortisone or fludrocortisone are preferred.

COVID-19, treatment

COVID-19, treatment: Very limited data available:

Note: Safety and effectiveness of dexamethasone or other corticosteroids for COVID-19 treatment have not been sufficiently evaluated in pediatric patients; use is extrapolated from adult patients; use caution. Reserve use for hospitalized patients who require high-flow oxygen, noninvasive ventilation, invasive mechanical ventilation, or extracorporeal membrane oxygenation (ECMO); not routinely recommended for pediatric patients requiring low levels of oxygen support (ie, nasal canula only). Use in profoundly immunocompromised pediatric patients should be considered only on a case-by-case basis as it has not been evaluated and may be harmful. Pediatric patients with COVID-19 should be enrolled in clinical trials whenever possible (Ref).

Infants, Children, and Adolescents: IV, Oral: 0.15 to 0.3 mg/kg/dose once daily for up to 10 days; maximum dose: 6 mg/dose (Ref). **Note:** An equivalent dose of an alternative glucocorticoid may be substituted if dexamethasone is unavailable (Ref).

Croup, mild to severe

Croup, mild to severe: Limited data available:

Infants and Children: Oral, IM, IV: 0.6 mg/kg once; maximum dose: 16 mg/dose, doses up to 20 mg/dose have been reported with similar efficacy; **Note:** Lower doses may be used in children with mild croup (0.15 to 0.6 mg/kg once) (Ref).

Dosage adjustment for concomitant therapy: Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Dosing: Kidney Impairment: Pediatric

Infants, Children, and Adolescents: IM, IV, Oral:

Kidney impairment: There are no dosage adjustments provided in the manufacturer's labeling; use with caution.

Hemodialysis or peritoneal dialysis: Based on adult data, supplemental dose is not necessary (Ref).

Dosing: Liver Impairment: Pediatric

Infants, Children, and Adolescents: There are no dosage adjustments provided in the manufacturer's labeling.

Adverse Reactions (Significant): Considerations

Adrenal suppression (tertiary adrenal insufficiency)

Adrenal suppression (tertiary adrenal insufficiency) may occur with glucocorticoids, including dexamethasone, and results from inadequate stimulation of the adrenal glands (Ref). Glucocorticoid-induced adrenal insufficiency usually resolves with discontinuation of dexamethasone, but symptoms may persist for 6 to 12 months (Ref). Adrenal insufficiency may lead to adrenal crisis, a life-threatening emergency that may present like a hypotensive shock state (Ref).

Mechanism: Dose- and time-related; occurs due to lack of or diminished cortisol production by the adrenal gland (Ref). Exogenous glucocorticoids produce a similar negative feedback mechanism as endogenous cortisol, causing a subsequent decrease in adrenocorticotrophic hormone (ACTH) secretion; thus, cortisol production is suppressed resulting in adrenal atrophy and subsequent insufficiency (ie, hypothalamic-pituitary-adrenal axis [HPA axis] suppression) (Ref). In times of stress (eg, critical illness, trauma, surgery), the body requires stress doses in patients taking dexamethasone chronically (Ref). Primary adrenal insufficiency can be caused by dexamethasone alone (without fludrocortisone) because of its lack of mineralocorticoid activity (Ref).

Onset: Varied; acute (minutes after administration) and/or chronic (2 to 20 hours to days) (Ref). Chronic dexamethasone use does not allow for the HPA axis to recover quickly (Ref).

Risk factors:

- Higher doses (eg, dexamethasone ≥ 0.25 mg daily) for prolonged periods (eg, ≥ 3 weeks) (Ref)
- Glucocorticoid potency (dexamethasone: high risk) (Ref)
- Route of glucocorticoid administration (eg, systemic: high risk) (Ref)
- Concurrent use of other glucocorticoids delivered by various routes of administration (eg, inhaled, topical, intraarticular injections) (Ref)
- History of previous adrenal crisis (Ref)
- Higher BMI (Ref)
- Older adults (Ref)

CNS and psychiatric/behavioral effects

Glucocorticoids, including dexamethasone, may cause a myriad of CNS and psychiatric/behavioral adverse reactions (Ref). Patients may develop excitatory **psychiatric disturbances** (including agitation, anxiety, distractibility, fear, hypomania, insomnia, irritability, lethargy, labile mood, mania, pressured speech, restlessness, and tearfulness) (Ref). Patients may also develop **apathy** or **depression**. Severe psychiatric effects, depression, and mania have been reported in adults receiving high-dose regimens (Ref). Discontinuation or dose reductions generally resolve symptoms over days to weeks (Ref).

Mechanism: Dose-related; not clearly established. Dexamethasone and other glucocorticoids may alter feedback on the hypothalamic-pituitary-adrenal axis, which may lead to mood changes (Ref). Glucocorticoids may induce glutamate release, which may be responsible for neuronal toxicity (Ref). Exogenous glucocorticoids may also impact GABAergic steroids (Ref).

Onset: Varied; most reactions occur early in therapy (ie, within 1 to 2 weeks after inhalation). May also occur later or after therapy discontinuation (Ref).

Risk factors:

- Higher doses (comparable to ≥ 80 mg prednisone) (Ref)

Possible additional risk factors:

- Age >30 years (Ref)
- Females (Ref)
- History of neuropsychiatric disorders (Ref)

Cushingoid features/Cushing syndrome

Glucocorticoids may cause a **cushingoid appearance** (truncal obesity, facial adipose tissue, dorsocervical adipose tissue), which are adverse reactions related to patient's physical features (Ref). Reactions are more metabolic than **weight gain**, which is related to **fluid retention (edema)** (Ref). Iatrogenic **Cushing syndrome** resulting from glucocorticoid therapy increases morbidity and mortality and decreases quality of life (Ref).

Mechanism: Dose- and time-related; excess cortisol from exogenous source (dexamethasone) results in suppression of adrenocorticotrophic hormone (ACTH), commonly called iatrogenic **Cushing syndrome** (Ref).

Onset: Delayed; may develop within the first 2 months of dexamethasone therapy, with the risk dependent on the dose and duration of treatment (Ref).

Risk factors:

- Higher doses (Ref)
- Longer duration of use (Ref)
- Drug interactions prolonging the half-life of glucocorticoids via cytochrome P450 (Ref)
- BMI (high) (Ref)
- Daily caloric intake (>30 kcal/kg/day) (Ref)

GI effects

Glucocorticoids, including dexamethasone, may cause GI effects, including **peptic ulcer** (with possible perforation and hemorrhage), dyspepsia, gastritis, **abdominal distention**, and **ulcerative esophagitis** (Ref). Meta-analyses suggest that glucocorticoid monotherapy carries little to no risk of peptic ulcer disease in the general population (Ref). Studies have demonstrated an increased risk of focal small bowel perforation in infants with low birth weight receiving dexamethasone (Ref).

Mechanism: Dose-related; glucocorticoids inhibit gastroprotective prostaglandin synthesis and reduce gastric mucus and bicarbonate secretion (Ref). Glucocorticoid immunosuppressive effects may prevent wound healing as well as mask GI signs and symptoms (Ref). Focal small bowel perforation is thought to be due to segmental degeneration of the muscularis externa (Ref).

Risk factors:

- Higher doses (equivalent to methylprednisolone ≥ 4 mg/day) (Ref)
- Concurrent aspirin or nonsteroidal anti-inflammatory drugs (NSAIDs) (Ref)
- Hospitalized (but not ambulatory) patients (Ref)
- Infants with low birth weight (Ref)
- Recent glucocorticoid users (7 to 28 days) versus remote or nonusers (Ref)

Hyperglycemia

Glucocorticoids, including dexamethasone, may provoke new-onset **hyperglycemia** in patients without a history of diabetes and may cause an **exacerbation of diabetes mellitus** (Ref). Glucose levels have been noted to increase 68% above baseline (Ref). Certain patient populations (eg, transplant, cancer, chronic rheumatologic conditions) are at particular risk due to medication combinations (Ref). Resolution may occur within 24 to 36 hours after dexamethasone discontinuation (Ref).

Mechanism: Dose- and time-related; increased insulin resistance (Ref). May also interfere with insulin signaling by direct effects on the insulin receptor and the glucose transporter and may promote gluconeogenesis via liver stimulation (Ref).

Onset: Rapid; 4 hours (Ref). Rapid onset of steroid-induced hyperglycemia occurred within 2 days after initiation of glucocorticoids with a peak in the late afternoon following daily dosing in the morning (Ref).

Risk factors:

- Dose and type of glucocorticoid (Ref)
- Duration of use (Ref)
- Divided versus once-daily dosing (Ref)
- IV and oral routes of administration (Ref)
- Older age (Ref)
- Males (Ref)
- BMI >25 kg/m² (Ref)
- African American or Hispanic (Ref)
- eGFR <40 mL/minute/1.73 m² (Ref)
- HbA_{1c} $\geq 6\%$ (Ref)
- History of gestational diabetes (Ref)
- Family history of diabetes mellitus (Ref)
- Concurrent use of mycophenolate mofetil and calcineurin inhibitors (Ref)
- Previous history of impaired fasting glucose or impaired glucose tolerance (Ref)
- Patients receiving palliative care (Ref)

Infection

Glucocorticoids, including dexamethasone, suppress the immune system and increase the risk of infection to any pathogen; immunosuppressive effects are reversible with discontinuation. Glucocorticoids may increase the risk to new infections, exacerbate existing infections, increase the risk of disseminated infections, increase the risk of reactivation or exacerbation of latent infections, and/or mask some signs of infections. **Infection** may occur after prolonged use and may include *Pneumocystis jirovecii pneumonia* (PJP), herpes zoster, tuberculosis, and other more common bacterial infections (Ref). Other infections may include fungal, helminthic, and protozoal infections. Some infections may be severe and life-threatening.

Mechanism: Dose- and time-related; related to pharmacologic action (ie, multiple activities on cell macrophage production and differentiation, inhibition of T-cell activation, and effects on dendritic cells) (Ref).

Onset: Varied; in one study, the median duration of glucocorticoid use prior to PJP diagnosis was 12 weeks but also occurred earlier or later in some cases (Ref).

Risk factors:

- Higher dose and longer duration of glucocorticoid (Ref); however, may also increase risk at lower doses (eg, ≤ 5 mg/day of prednisone or equivalent) (Ref)
- Immunocompromised state (Ref)
- Concurrent medications (immunosuppressive) (Ref)
- Rheumatoid arthritis (Ref)
- Interstitial lung disease (Ref)
- Older adults (Ref)
- Male (Ref)
- Low performance status (Ref)

Neuromuscular and skeletal effects

Glucocorticoid (including dexamethasone)-induced neuromuscular and skeletal effects can take the form of various pathologies in patients ranging from **osteoporosis** and **vertebral compression fracture to myopathy to osteonecrosis** in adult and pediatric patients (Ref). Glucocorticoid use is the most common cause of secondary osteoporosis; may be underrecognized and undertreated due to underestimation of risk in this patient population (Ref). Vertebral fractures are the most common glucocorticoid-related fracture (Ref). Myopathies can also occur secondary to direct skeletal muscle catabolism (Ref). Acute **steroid myopathy** is rare (Ref).

Mechanism: Dose- and time-related; glucocorticoids have direct/indirect effects on bone remodeling with osteoblast recruitment decreasing and apoptosis increasing (Ref). Myopathies or **myasthenia** result from reductions in protein synthesis and protein catabolism, which can manifest as proximal muscle weakness and atrophy in the upper and lower extremities (Ref).

Onset: Delayed; vertebral fracture risk is increased within 3 months of initiation and peaks at 12 months (Ref).

Risk factors:

Drug-related risks:

- Cumulative dose of glucocorticoids prednisone > 5 g or equivalent (Ref)
- Children receiving ≥ 4 courses of glucocorticoids (Ref)
- Prednisone ≥ 2.5 mg to 7.5 mg daily or equivalent for ≥ 3 months (Ref)
- Myopathy may occur at prednisone doses ≥ 10 mg daily or equivalent, with higher doses potentiating more of a rapid onset (Ref)
- Fluorinated glucocorticoid preparations (eg, dexamethasone, betamethasone, triamcinolone) have a higher risk of myopathies (Ref)

General fracture risks:

- Age > 55 years (Ref)
- BMI < 18.5 kg/m² (Ref)
- Bone mineral T score below -1.5 (Ref)
- Endocrine disorders (eg, hypogonadism, hyper- or hypoparathyroidism) (Ref)

- Excess alcohol use (>2 units/day) (Ref)
- Females (Ref)
- History of falls (Ref)
- Malabsorption (Ref)
- Menopause and duration of menopause (Ref)
- White race (Ref)
- Patients with cancer (Ref)
- Previous fracture (Ref)
- Smoking (Ref)
- Underlying inflammatory condition in all ages (eg, inflammatory bowel disease, rheumatoid arthritis) (Ref)

Ocular effects

Glucocorticoid (including dexamethasone)-induced ocular effects may include **increased intraocular pressure** (IOP), **glaucoma** (open-angle), and **subcapsular posterior cataract** in adult and pediatric patients (Ref). Cataracts may persist after discontinuation of glucocorticoid therapy (Ref).

Mechanism: Dose- and time-related; Glucocorticoids can induce cataracts by covalently bonding to lens proteins, causing destabilization of the protein structure, and oxidative changes leading to cataracts formation (Ref). There are various proposed mechanisms of IOP contributing to glaucoma, including accumulation of polymerized glycosaminoglycans in the trabecular meshwork, producing edema and increasing outflow resistance (Ref). Another mechanism may include inhibition of phagocytic endothelial cells, leading to accumulation of aqueous debris (Ref). Glucocorticoids can also alter the trabecular meshwork causing an increase in nuclear size and DNA content (Ref). In addition, they can decrease the synthesis of prostaglandins which regulate the aqueous outflow (Ref).

Onset: Delayed; cataracts may occur at least 1 year after initiation of chronic glucocorticoid therapy (Ref). IOP may occur at 4 years or more after initiation (Ref).

Risk factors:

- Dose (Ref)
- Topical > Systemic (Ref)
- Duration of use in all ages (Ref)
- Family history of open-angle glaucoma (Ref)
- Type I diabetes mellitus (Ref)
- High myopia (Ref)
- Pseudophakia (Ref)
- Prior vitrectomies (Ref)
- Connective tissue disease and sex (eg, rheumatoid arthritis in males) (Ref)
- Older patients or age <6 years (Ref)
- Genetics (Ref)
- Angle recessive glaucoma (Ref)

Adverse Reactions

The following adverse drug reactions and incidences are derived from product labeling unless otherwise specified. Some reactions listed are based on reports for other agents in this same pharmacologic class and may not be specifically reported for dexamethasone.

Frequency not defined:

Cardiovascular: Bradycardia, cardiac arrhythmia, cardiomegaly, circulatory shock, edema, embolism (fat), heart failure (in susceptible patients), hypertension, myocardial rupture (after recent myocardial infarction), syncope, tachycardia, thromboembolism, thrombophlebitis, vasculitis

Dermatologic: Acne vulgaris, allergic dermatitis, alopecia, atrophic striae, diaphoresis, ecchymoses, erythema of skin, facial erythema, fragile skin, hyperpigmentation, hypertrichosis, hypopigmentation, inadvertent suppression of skin test reaction, skin atrophy, skin rash, subcutaneous atrophy, urticaria, xeroderma

Endocrine & metabolic: Decreased serum potassium, fluid retention, growth suppression (children), hirsutism, hypokalemic alkalosis, menstrual disease, negative nitrogen balance (due to protein catabolism), sodium retention, weight gain

Gastrointestinal: Hiccups, increased appetite, nausea, pancreatitis, pruritus ani (following IV injection)

Genitourinary: Defective spermatogenesis (increased or decreased), glycosuria

Hematologic & oncologic: Petechia

Hepatic: Hepatomegaly, increased serum transaminases

Hypersensitivity: Anaphylaxis, angioedema, nonimmune anaphylaxis

Infection: Sterile abscess

Local: Postinjection flare (intra-articular use)

Nervous system: Amyotrophy, emotional lability, euphoria, headache, increased intracranial pressure, intracranial hypertension (idiopathic; usually following discontinuation), malaise, myasthenia, neuritis, neuropathy, paresthesia, personality changes, seizure, vertigo

Neuromuscular & skeletal: Charcot arthropathy, rupture of tendon

Ophthalmic: Exophthalmos

Respiratory: Pulmonary edema

Miscellaneous: Wound healing impairment

Postmarketing:

Cardiovascular: Hypertrophic cardiomyopathy (premature infants) (Ref)

Dermatologic: Perianal skin irritation (itching, burning, tingling; following rapid IV injection; more common in females, with higher doses; sudden onset with resolution in <1 minute) (Ref)

Endocrine & metabolic: Adrenal suppression (tertiary) (Ref), Cushing syndrome (iatrogenic) (Ref), cushingoid appearance (Ref), exacerbation of diabetes mellitus (Ref), hyperglycemia (Ref), impaired glucose tolerance (Ref), moon face (Ref), redistribution of body fat (Ref)

Gastrointestinal: Abdominal distention (Ref), intestinal perforation (Ref), peptic ulcer (with possible perforation and hemorrhage) (Ref), ulcerative esophagitis (Ref)

Hematologic & oncologic: Kaposi sarcoma (Ref), tumor lysis syndrome (Ref)

Infection: Infection (Ref)

Nervous system: Apathy (Ref), depression (Ref), psychiatric disturbance (including agitation, anxiety, distractibility, euphoria, fear, hypomania, insomnia, irritability, labile mood, lethargy, pressured speech, restlessness, tearfulness) (Ref)), paralysis (hypokalemic periodic paralysis (Ref)

Neuromuscular & skeletal: Bone fracture (Ref), myopathy (Ref), osteonecrosis (femoral and humeral heads) (Ref), osteoporosis (Ref), steroid myopathy (Ref), vertebral compression fracture (Ref)

Ophthalmic: Glaucoma (Ref), increased intraocular pressure (Ref), subcapsular posterior cataract (Ref)

Contraindications

Hypersensitivity to dexamethasone or any component of the formulation; systemic fungal infections.

Canadian labeling: Additional contraindications (not in US labeling):

Injection: Bacteremia; gastric and duodenal ulcers; glaucoma; Cushing syndrome; certain viral infections (ie, varicella, herpes genitalis); administration to live virus vaccines.

Oral: Gastric and duodenal ulcers; infection with tropical worms; active or suspected ocular or periocular infections; advanced glaucoma in patients with cup to disk ratios >0.8; systemic infection unless specific anti-infective therapy is employed.

Warnings/Precautions

Concerns related to adverse effects:

- Adrenal suppression: May cause hypercortisolism or suppression of hypothalamic-pituitary-adrenal axis, particularly in younger children.
- Anaphylactoid reactions: Rare cases of anaphylactoid reactions have been observed in patients receiving corticosteroids.
- Kaposi sarcoma: Prolonged treatment with corticosteroids has been associated with the development of Kaposi sarcoma; clinical improvement may occur with dexamethasone discontinuation.

Disease-related concerns:

- Adrenal insufficiency: Dexamethasone does not provide any mineralocorticoid activity in adrenal insufficiency (may be employed as a single dose while cortisol assays are performed). Hydrocortisone is the preferred treatment of chronic primary adrenal insufficiency and adrenal crisis (ES [Bornstein 2016]).
- Cardiovascular disease: Use with caution in patients with heart failure and/or hypertension; use has been associated with fluid retention, electrolyte disturbances, and hypertension. Monitor BP. Use with caution following acute myocardial infarction; corticosteroids have been associated with myocardial rupture.
- GI disease: Use with caution in patients with GI diseases (diverticulitis, fresh intestinal anastomoses, active or latent peptic ulcer, ulcerative colitis, abscess, or other pyogenic infection) due to GI perforation risk. Signs of GI perforation may be masked in patients receiving corticosteroid therapy.
- Head injury: Increased mortality was observed in patients receiving high-dose IV methylprednisolone. High-dose corticosteroids should not be used for the management of head injury (BTF [Carney 2016]).
- Hepatic impairment: Use with caution in patients with hepatic impairment, including cirrhosis; long-term use has been associated with fluid retention.
- Hepatitis B: Reactivation may occur; consider consultation with health care provider experienced with hepatitis B in patients who show evidence of hepatitis B infection.
- Myasthenia gravis: Use may cause transient worsening of myasthenia gravis (MG) (eg, within first 2 weeks of treatment); monitor for worsening MG (AAN [Narayanaswami 2021]).
- Ocular disease: Use with caution in patients with a history of ocular herpes simplex; corneal perforation has occurred; do not use in active ocular herpes simplex. Not recommended for the treatment of optic neuritis; may increase frequency of new episodes.
- Pheochromocytoma: Pheochromocytoma crisis (may be fatal) has been reported after administration of systemic corticosteroids. Consider the risk of pheochromocytoma crisis in patients with suspected or confirmed pheochromocytoma.
- Renal impairment: Use with caution in patients with renal impairment; fluid retention may occur.
- Seizure disorders: Use corticosteroids with caution in patients with a history of seizure disorder; seizures have been reported with adrenal crisis.

- Systemic sclerosis: Use with caution in patients with systemic sclerosis; an increase in scleroderma renal crisis incidence has been observed with corticosteroid use. Monitor BP and renal function in patients with systemic sclerosis treated with corticosteroids (EULAR [Kowal-Bielecka 2017]).
- Thyroid disease: Changes in thyroid status may necessitate dosage adjustments; metabolic clearance of corticosteroids increases in patients with hyperthyroidism and decreases in patients with hypothyroidism.

Concurrent drug therapy issues:

- Immunizations: Avoid administration of live or live attenuated vaccines in patients receiving immunosuppressive doses of corticosteroids. Non-live or inactivated vaccines may be administered, although the response cannot be predicted.

Special populations:

- Older adult: Use with caution with the smallest possible effective dose for the shortest duration in older adults.
- Pediatric: May affect growth velocity; growth should be routinely monitored in pediatric patients.

Dosage form specific issues:

- Benzyl alcohol and derivatives: Some dosage forms may contain sodium benzoate/benzoic acid; benzoic acid (benzoate) is a metabolite of benzyl alcohol; large amounts of benzyl alcohol (≥ 99 mg/kg/day) have been associated with a potentially fatal toxicity ("gasping syndrome") in neonates; the "gasping syndrome" consists of metabolic acidosis, respiratory distress, gasping respirations, CNS dysfunction (including convulsions, intracranial hemorrhage), hypotension, and cardiovascular collapse (AAP ["Inactive" 1997]; CDC 1982); some data suggests that benzoate displaces bilirubin from protein binding sites (Ahlfors 2001); avoid or use dosage forms containing benzyl alcohol derivative with caution in neonates. See manufacturer's labeling.
- Ethanol: Oral concentrate (Intensol) contains 30% v/v ethyl alcohol.
- Propylene glycol: Some dosage forms may contain propylene glycol; large amounts are potentially toxic and have been associated hyperosmolality, lactic acidosis, seizures, and respiratory depression; use caution (AAP ["Inactive" 1997]; Zar 2007).
- Sulfite: Some products may contain sodium sulfite, a sulfite that may cause allergic-type reactions including anaphylaxis and life-threatening or less severe asthmatic episodes in susceptible patients.

Other warnings/precautions:

- Discontinuation of therapy: Withdraw therapy with gradual tapering of dose.
- Epidural injection: Corticosteroids are not approved for epidural injection. Serious neurologic events (eg, spinal cord infarction, paraplegia, quadriplegia, cortical blindness, stroke), some resulting in death, have been reported with epidural injection of corticosteroids, with and without use of fluoroscopy.
- Intra-articular injection: May produce systemic as well as local effects. Appropriate examination of any joint fluid present is necessary to exclude a septic process. Avoid injection into an infected site. Do not inject into unstable joints. Patients should not overuse joints in which symptomatic benefit has been obtained as long as the inflammatory process remains active. Frequent intra-articular injection may result in damage to joint tissues.
- Stress: Patients may require higher doses when subject to stress (ie, trauma, surgery, severe infection).

Warnings: Additional Pediatric Considerations

In premature neonates, the use of high-dose dexamethasone (approximately >0.5 mg/kg/day) for the prevention or treatment of bronchopulmonary dysplasia has been associated with adverse neurodevelopmental outcomes, including higher rates of cerebral palsy without additional clinical benefit over

lower doses; current data do not support use of high doses; further studies are needed (Watterberg 2010).

Some dosage forms may contain propylene glycol; in neonates, large amounts of propylene glycol delivered orally, intravenously (eg, >3,000 mg/day), or topically have been associated with potentially fatal toxicities, which can include metabolic acidosis, seizures, renal failure, and CNS depression; toxicities have also been reported in children and adults, including hyperosmolality, lactic acidosis, seizures, and respiratory depression; use caution (AAP 1997; Shehab 2009).

Dosage Forms: US

Excipient information presented when available (limited, particularly for generics); consult specific product labeling. [DSC] = Discontinued product

Concentrate, Oral:

æxAMETHasone Intensol: 1 mg/mL (30 mL) [contains alcohol, usp; unflavored flavor]

Elixir, Oral:

Generic: 0.5 mg/5 mL (237 mL)

Kit, Injection, as sodium phosphate:

ReadySharp æxAMETHasone: 10 mg/mL [DSC] [contains benzyl alcohol, sodium sulfite]

TopiDex: 10 mg/mL [contains benzyl alcohol]

Kit, Injection, as sodium phosphate [preservative free]:

DoubleDex: 10 mg/mL

MAS Care-Pak: 10 mg/mL

Solution, Oral:

Generic: 0.5 mg/5 mL (240 mL, 500 mL)

Solution, Injection, as sodium phosphate:

Generic: 4 mg/mL (1 mL); 20 mg/5 mL (5 mL); 120 mg/30 mL (30 mL); 10 mg/mL (1 mL); 100 mg/10 mL (10 mL)

Solution, Injection, as sodium phosphate [preservative free]:

Generic: 10 mg/mL (1 mL)

Solution Prefilled Syringe, Injection, as sodium phosphate [preservative free]:

Generic: 4 mg/mL (1 mL); 10 mg/mL (1 mL)

Tablet, Oral:

Hemaay: 20 mg [contains corn starch]

Generic: 0.5 mg, 0.75 mg, 1 mg, 1.5 mg, 2 mg, 4 mg, 6 mg

Tablet Therapy Pack, Oral:

Dexabliss: 1.5 MG (39) (39 ea [DSC])

Dxevo 11-Day: 1.5 mg (39 ea [DSC])

HiDex 6-Day: 1.5 mg (21 ea) [scored; contains fd&c red #40 (allura red ac dye)]

TaperDex 12-Day: 1.5 mg (49 ea) [contains fd&c red #40 (allura red ac dye)]

TaperDex 6-Day: 1.5 mg (21 ea) [scored; contains fd&c red #40 (allura red ac dye)]

TaperDex 6-Day: 1.5 mg (21 ea) [scored; contains fd&c red #40(allura red ac)aluminum lake]

TaperDex 7-Day: 1.5 mg (27 ea) [scored; contains fd&c red #40 (allura red ac dye)]

ZCORT 7-Day: 1.5 mg (25 ea [DSC])

Generic: 1.5 mg (21 ea, 35 ea, 51 ea)

Generic Equivalent Available: US

May be product dependent

Pricing: US

Concentrate (dexamETHasone Intensol Oral)

1 mg/mL (per mL): \$1.07

Elixir (dexamETHasone Oral)

0.5 mg/5 mL (per mL): \$0.27 - \$1.77

Kit (DoubleDex Injection)

10 mg/mL (per each): \$598.00

Kit (MAS Care-Pak Injection)

10 mg/mL (per each): \$605.00

Solution (dexamETHasone Oral)

0.5 mg/5 mL (per mL): \$0.27

Solution (dexamETHasone Sod Phosphate PF Injection)

10 mg/mL (per mL): \$1.49 - \$9.00

Solution (dexamETHasone Sodium Phosphate Injection)

4 mg/mL (per mL): \$0.82 - \$3.46

10 mg/mL (per mL): \$1.49 - \$1.72

20 mg/5 mL (per mL): \$0.21 - \$1.66

100 mg/10 mL (per mL): \$0.48 - \$1.86

120 mg/30 mL (per mL): \$0.39 - \$1.86

Solution Prefilled Syringe (dexamETHasone Sod Phos (PF) Injection)

10 mg/mL (per mL): \$6.94

Tablet Therapy Pack (dexamETHasone Oral)

1.5MG (21) (per each): \$8.06 - \$8.54

1.5MG (35) (per each): \$8.55

1.5MG (51) (per each): \$8.54

Tablet Therapy Pack (HiDex 6-Day Oral)

1.5MG (21) (per each): \$33.29

Tablet Therapy Pack (TaperDex 12-Day Oral)

1.5MG (49) (per each): \$5.68

Tablet Therapy Pack (TaperDex 6-Day Oral)

1.5 mg (per each): \$10.80

1.5MG (21) (per each): \$10.80

Tablet Therapy Pack (TaperDex 7-Day Oral)

1.5MG (27) (per each): \$8.66

Tablets (dexamETHasone Oral)

0.5 mg (per each): \$0.14 - \$0.21

0.75 mg (per each): \$0.25 - \$0.27

1 mg (per each): \$0.30 - \$0.37

1.5 mg (per each): \$0.48 - \$15.95

2 mg (per each): \$0.55 - \$0.96

4 mg (per each): \$1.19 - \$1.25

6 mg (per each): \$1.78 - \$2.15

Tablets (Hemaady Oral)

20 mg (per each): \$35.96

Disclaimer: A representative AWP (Average Wholesale Price) price or price range is provided as reference price only. A range is provided when more than one manufacturer's AWP price is available and uses the low and high price reported by the manufacturers to determine the range. The pricing data should be used for benchmarking purposes only, and as such should not be used alone to set or adjudicate any prices for reimbursement or purchasing functions or considered to be an exact price for a single product and/or manufacturer. Medi-Span expressly disclaims all warranties of any kind or nature, whether express or implied, and assumes no liability with respect to accuracy of price or price range data published in its solutions. In no event shall Medi-Span be liable for special, indirect, incidental, or consequential damages arising from use of price or price range data. Pricing data is updated monthly.

Dosage Forms: Canada

Excipient information presented when available (limited, particularly for generics); consult specific product labeling.

Elixir, Oral:

Generic: 0.5 mg/5 mL (100 ea, 100 mL)

Solution, Injection, as sodium phosphate:

Generic: 4 mg/mL (5 mL); 10 mg/mL (1 mL, 10 mL)

Tablet, Oral:

Generic: 0.5 mg, 0.75 mg, 2 mg, 4 mg

Extemporaneous Preparations

Note: In the United States, dexamethasone is commercially available as an oral solution (0.1 mg/mL) and as an oral concentrate (1 mg/mL).

1 mg/mL Oral Suspension

A 1 mg/mL oral suspension may be made using dexamethasone tablets and either Oral Mix **or** Oral Mix SF. Crush twenty-five 4 mg tablets in a mortar and reduce to a fine powder. Add a small amount of the chosen vehicle and mix to a uniform paste; mix while adding the vehicle in incremental amounts to **almost** 100 mL; transfer to a calibrated, amber bottle, rinse mortar with vehicle, and add sufficient quantity of vehicle to make 100 mL. Label "shake well." May be stored for up to 91 days at room temperature or refrigerated (Ref).

Administration: Adult

Oral: Administer with meals to help prevent GI upset. If appropriate, may administer antacids between meals to help prevent peptic ulcers.

Oral concentrate: Use only the calibrated dropper provided. Draw dose into dropper; squeeze dropper contents into a liquid or semi-solid food (water, juice, soda or soda-like beverage, applesauce,

pudding). Gently stir for a few seconds. Administer the entire mixture immediately. Do not store for future use.

IV: May administer 4 mg/mL or 10 mg/mL concentration undiluted over ≥ 1 minute (Ref) or infuse by IVPB over 5 to 30 minutes (Ref). Rapid administration of dexamethasone may be associated with perineal irritation (especially with higher doses); consider further dilution and slower administration by IVPB to avoid perineal irritation (Ref).

IM: Administer 4 mg/mL or 10 mg/mL concentration by deep IM injection.

Intra-articular or soft tissue injection: Note: Dexamethasone **sodium phosphate** (a short-acting solution) is the only formulation available for intra-articular or soft tissue injections in the United States or Canada. Other glucocorticoids, such as triamcinolone acetonide or methylprednisolone acetate, are used more commonly for intra-articular or soft tissue injection (Ref). Refer to product-specific labeling for further details.

Intra-articular: Administer into affected joint.

Soft tissue: Administer into affected tissue.

Intralesional injection: Note: Dexamethasone **sodium phosphate** (a short-acting solution) is the only formulation available for intralesional injections in the United States or Canada. Another glucocorticoid, triamcinolone acetonide, is used more commonly for intralesional injection (Ref). Refer to product-specific labeling for further details.

Intralesional: Administer into affected area.

Preparation for Administration: Adult

IV: May be given undiluted or further diluted in NS or D5W. For administration via IVPB, dexamethasone dilutions in 50 mL of NS have been described (Ref).

Administration: Pediatric

Oral: May administer with food or milk to decrease GI adverse effects.

Parenteral: Use preservative-free dosage forms in neonates.

IM: May administer 4 mg/mL or 10 mg/mL undiluted.

IV: May administer as undiluted solution (4 mg/mL or 10 mg/mL) slow IV push, usually over 1 to 4 minutes; rapid administration is associated with perineal discomfort (burning, tingling) (Ref); may consider further dilution of high doses and administration by IV intermittent infusion over 15 to 30 minutes (Ref).

Preparation for Administration: Pediatric

Oral: Oral administration of parenteral solution (Ref): In some pediatric croup and asthma exacerbation trials, doses have been prepared using a parenteral dexamethasone formulation (10 mg/mL) and mixing it with an oral flavored syrup (Ref).

IV: May be administered undiluted (4 mg/mL or 10 mg/mL). High doses may be further diluted in NS or D5W (Ref). In neonates, use a preservative-free product. In adults, dexamethasone dilutions in 50 mL of NS have been described (Ref).

Storage/Stability

Elixir: Store at 15°C to 30°C (59°F to 86°F); avoid freezing.

Injection: Store intact vials at 20°C to 25°C (68°F to 77°F). Protect from light, heat, and freezing. Do not autoclave. Diluted solutions should be used within 24 hours.

Oral concentrated solution (Intensol): Store at 20°C to 25°C (68°F to 77°F); do not freeze; do not use if precipitate is present; dispense only in original bottle and only with manufacturer-supplied calibrated dropper; discard open bottle after 90 days.

Oral solution: Store at 20°C to 25°C (68°F to 77°F); protect from moisture.

Tablets: Store at 20°C to 25°C (68°F to 77°F); excursions permitted to 15°C to 30°C (59°F to 86°F). Dispense in a tight, light-resistant, child-resistant container. Protect from moisture.

Use: Labeled Indications

Oral, IV, or IM injection: Anti-inflammatory or immunosuppressant agent in the treatment of a variety of diseases, including those of allergic, hematologic (eg, immune thrombocytopenia), dermatologic, neoplastic (leukemias, lymphomas, and multiple myeloma), rheumatic, autoimmune, nervous system, renal, and respiratory origin; primary or secondary adrenocorticoid deficiency (not first line), congenital adrenal hyperplasia; management of shock, cerebral edema, and as a diagnostic agent.

Intra-articular or soft tissue injection: As adjunctive therapy for short-term administration in synovitis of osteoarthritis, rheumatoid arthritis, acute and subacute bursitis, acute gouty arthritis, epicondylitis, acute nonspecific tenosynovitis, and posttraumatic osteoarthritis.

Intralesional injection: Keloids; localized hypertrophic, infiltrated, inflammatory lesions of lichen planus, psoriatic plaques, granuloma annulare, and lichen simplex chronicus (neurodermatitis); discoid lupus erythematosus; necrobiosis lipoidica diabetorum; alopecia areata; cystic tumors of an aponeurosis or tendon (ganglia).

Use: Off-Label: Adult

Acute mountain sickness/high-altitude cerebral edema; Acute respiratory distress syndrome; Antiemetic regimens: Chemotherapy-associated nausea and vomiting, prevention; Antiemetic regimens: Postoperative nausea and vomiting, prevention; Antiemetic regimens: Radiation therapy-associated nausea and vomiting, prevention; Asthma, acute exacerbation; Cancer-related cachexia; COVID-19, hospitalized patients; Fetal lung maturation, acceleration of; Meningitis (bacterial), prevention of neurologic complications; Migraine, recurrence prevention

Medication Safety Issues

Sound-alike/look-alike issues:

DexAMETHasone may be confused with desoximetasone, dexmedetomidine, dextroamphetamine

Older Adult: High-Risk Medication:

Dexamethasone is identified in the Screening Tool of Older Person's Prescriptions (STOPP) criteria as a potentially inappropriate medication in older adults (≥ 65 years of age) as long-term monotherapy (>3 months) for rheumatoid arthritis or osteoarthritis treatment (unless as periodic intra-articular use). In addition, some disease states of concern include heart failure, peptic ulcer disease, erosive esophagitis, and chronic obstructive pulmonary disease (inhaled therapies preferred) (O'Mahony 2023).

Metabolism/Transport Effects

Substrate of CYP3A4 (Major), P-glycoprotein (Minor); **Note:** Assignment of Major/Minor substrate status based on clinically relevant drug interaction potential; **Induces** CYP3A4 (Weak)

Drug Interactions

(For additional information: Launch drug interactions program)

Note: Interacting drugs may **not be individually listed below** if they are part of a group interaction (eg, individual drugs within "CYP3A4 Inducers [Strong]" are NOT listed). For a complete list of drug interactions by individual drug name and detailed management recommendations, use the drug interactions program by clicking on the "Launch drug interactions program" link above.

Abrocitinib: Corticosteroids (Systemic) may increase immunosuppressive effects of Abrocitinib. Management: The use of abrocitinib in combination with other immunosuppressants is not recommended. Doses equivalent to more than 2 mg/kg or 20 mg/day of prednisone (for persons over 10 kg) administered for 2 or more weeks are considered immunosuppressive. *Risk D: Consider Therapy Modification*

Acetylcholinesterase Inhibitors: Corticosteroids (Systemic) may increase adverse/toxic effects of Acetylcholinesterase Inhibitors. Increased muscular weakness may occur. *Risk C: Monitor*

Aldesleukin: Corticosteroids (Systemic) may decrease therapeutic effects of Aldesleukin. *Risk X: Avoid*

Amphotericin B: Corticosteroids (Systemic) may increase hypokalemic effects of Amphotericin B. *Risk C: Monitor*

Androgens: Corticosteroids (Systemic) may increase fluid-retaining effects of Androgens. *Risk C: Monitor*

Antacids: May decrease bioavailability of DexAMETHasone (Systemic). *Risk C: Monitor*

Antidiabetic Agents: Hyperglycemia-Associated Agents may decrease therapeutic effects of Antidiabetic Agents. *Risk C: Monitor*

Antithymocyte Globulin (Equine): Corticosteroids (Systemic) may increase adverse/toxic effects of Antithymocyte Globulin (Equine). Specifically, these effects may be unmasked if the dose of systemic corticosteroid is reduced. Corticosteroids (Systemic) may increase immunosuppressive effects of Antithymocyte Globulin (Equine). Specifically, infections may occur with greater severity and/or atypical presentations. *Risk C: Monitor*

Aprepitant: May increase serum concentrations of DexAMETHasone (Systemic). Management: Reduce dexamethasone dose 50% with aprepitant. Aprepitant labeling incorporates this recommendation into the dose provided for dexamethasone; further reduction is not necessary. No dose adjustment may be needed with single, low-dose aprepitant for PONV. *Risk D: Consider Therapy Modification*

Atazanavir: May increase serum concentrations of DexAMETHasone (Systemic). DexAMETHasone (Systemic) may decrease serum concentrations of Atazanavir. Management: Consider alternatives to this combination due to the potential for loss of atazanavir efficacy and development of resistance. Monitor for decreased atazanavir efficacy and increased dexamethasone adverse effects with any combined used. *Risk D: Consider Therapy Modification*

Atogepant: CYP3A4 Inducers (Weak) may decrease serum concentrations of Atogepant. Management: For episodic migraine, the recommended dose of atogepant is 30 mg once daily or 60 mg once daily if used with weak CYP3A4 inducers. For treatment of chronic migraine, the dose of atogepant is 60 mg once daily if used with weak CYP3A4 inducers. *Risk D: Consider Therapy Modification*

Axitinib: DexAMETHasone (Systemic) may decrease serum concentrations of Axitinib. *Risk C: Monitor*

Baricitinib: Corticosteroids (Systemic) may increase immunosuppressive effects of Baricitinib. Management: The use of baricitinib in combination with potent immunosuppressants is not recommended. Doses equivalent to more than 2 mg/kg or 20 mg/day of prednisone (for persons over 10 kg) administered for 2 or more weeks are considered immunosuppressive. *Risk D: Consider Therapy Modification*

BCG Products: Corticosteroids (Systemic) may decrease therapeutic effects of BCG Products. Corticosteroids (Systemic) may increase adverse/toxic effects of BCG Products. Specifically, the risk of vaccine-associated infection may be increased. *Risk X: Avoid*

Brincidofovir: Corticosteroids (Systemic) may decrease therapeutic effects of Brincidofovir. *Risk C: Monitor*

Brivudine: May increase adverse/toxic effects of Corticosteroids (Systemic). *Risk X: Avoid*

Calcitriol (Systemic): Corticosteroids (Systemic) may decrease therapeutic effects of Calcitriol (Systemic). *Risk C: Monitor*

CAR-T Cell Immunotherapy: Corticosteroids (Systemic) may decrease therapeutic effects of CAR-T Cell Immunotherapy. Corticosteroids (Systemic) may increase adverse/toxic effects of CAR-T Cell Immunotherapy. Specifically, the severity and duration of neurologic toxicities may be increased.

Management: Avoid use of corticosteroids as premedication before treatment with CAR-T cell immunotherapy agents. Corticosteroids are indicated and may be required for treatment of toxicities such as cytokine release syndrome or neurologic toxicity. *Risk D: Consider Therapy Modification*

Cardiac Glycosides: Corticosteroids (Systemic) may increase adverse/toxic effects of Cardiac Glycosides. *Risk C: Monitor*

Caspofungin: Inducers of Drug Clearance may decrease serum concentrations of Caspofungin. Management: Consider increasing the caspofungin dose to 70 mg daily, or 70 mg/m², up to a maximum of 70 mg for pediatric patients, during coadministration with these inducers of drug clearance. *Risk D: Consider Therapy Modification*

Chikungunya Vaccine (Live): Corticosteroids (Systemic) may increase adverse/toxic effects of Chikungunya Vaccine (Live). Specifically, the risk of vaccine-associated infection may be increased. Corticosteroids (Systemic) may decrease therapeutic effects of Chikungunya Vaccine (Live). *Risk X: Avoid*

Cholestyramine Resin: May decrease serum concentrations of DexAMETHasone (Systemic). *Risk C: Monitor*

Cladribine: Corticosteroids (Systemic) may increase immunosuppressive effects of Cladribine. *Risk X: Avoid*

Clofazimine: May increase serum concentrations of CYP3A4 Substrates (High risk with Inhibitors). *Risk C: Monitor*

CloZAPine: CYP3A4 Inducers (Weak) may decrease serum concentrations of CloZAPine. *Risk C: Monitor*

Cobicistat: May increase serum concentrations of DexAMETHasone (Systemic). DexAMETHasone (Systemic) may decrease serum concentrations of Cobicistat. Management: Consider an alternative corticosteroid. Monitor patients receiving this combination closely for evidence of diminished response to the antiviral regimen as well as increased dexamethasone effects and toxicities. *Risk D: Consider Therapy Modification*

Coccidioides immitis Skin Test: Coadministration of Corticosteroids (Systemic) and Coccidioides immitis Skin Test may alter diagnostic results. Management: Consider discontinuing systemic corticosteroids (dosed at 2 mg/kg or 20 mg/day of prednisone (for persons over 10 kg) administered for 2 or more weeks) several weeks prior to coccidioides immitis skin antigen testing. *Risk D: Consider Therapy Modification*

Cosyntropin: Coadministration of Corticosteroids (Systemic) and Cosyntropin may alter diagnostic results. *Risk C: Monitor*

COVID-19 Vaccine (Inactivated Virus): Corticosteroids (Systemic) may decrease therapeutic effects of COVID-19 Vaccine (Inactivated Virus). *Risk C: Monitor*

COVID-19 Vaccine (mRNA): Corticosteroids (Systemic) may decrease therapeutic effects of COVID-19 Vaccine (mRNA). Management: Give a 3-dose primary series for all patients aged 6 months and older taking immunosuppressive medications or therapies. Booster doses are recommended for certain age groups. See CDC guidance for details. *Risk D: Consider Therapy Modification*

COVID-19 Vaccine (Subunit): Corticosteroids (Systemic) may decrease therapeutic effects of COVID-19 Vaccine (Subunit). *Risk C: Monitor*

CYP3A4 Inducers (Moderate): May decrease serum concentrations of DexAMETHasone (Systemic). *Risk C: Monitor*

CYP3A4 Inducers (Strong): May decrease serum concentrations of DexAMETHasone (Systemic). Management: Avoid coadministration of dexamethasone and strong CYP3A4 inducers. If concomitant use cannot be avoided, consider dexamethasone dose increases. *Risk D: Consider Therapy Modification*

CYP3A4 Inhibitors (Moderate): May increase serum concentrations of DexAMETHasone (Systemic). *Risk C: Monitor*

CYP3A4 Inhibitors (Strong): May increase serum concentrations of DexAMETHasone (Systemic). *Risk C: Monitor*

Daclatasvir: DexAMETHasone (Systemic) may decrease serum concentrations of Daclatasvir. *Risk X: Avoid*

Darunavir: May increase serum concentrations of DexAMETHasone (Systemic). DexAMETHasone (Systemic) may decrease serum concentrations of Darunavir. Management: Consider alternatives to this combination due to the potential for loss of darunavir efficacy and development of resistance. Monitor for decreased darunavir efficacy and increased dexamethasone adverse effects with any combined used. *Risk D: Consider Therapy Modification*

Deferasirox: Corticosteroids (Systemic) may increase adverse/toxic effects of Deferasirox. Specifically, the risk for GI ulceration/irritation or GI bleeding may be increased. *Risk C: Monitor*

Delgocitinib: May increase immunosuppressive effects of Corticosteroids (Systemic). Management: The use of delgocitinib in combination with other immunosuppressants is not recommended. Doses equivalent to more than 2 mg/kg or 20 mg/day of prednisone (for persons over 10 kg) administered for 2 or more weeks are considered immunosuppressive. *Risk D: Consider Therapy Modification*

Dengue Tetravalent Vaccine (Live): Corticosteroids (Systemic) may decrease therapeutic effects of Dengue Tetravalent Vaccine (Live). Corticosteroids (Systemic) may increase adverse/toxic effects of Dengue Tetravalent Vaccine (Live). Specifically, the risk of vaccine associated infection may be increased. *Risk X: Avoid*

Denosumab: May increase immunosuppressive effects of Corticosteroids (Systemic). Management: Consider the risk of serious infections versus the potential benefits of coadministration of denosumab and systemic corticosteroids. If combined, monitor patients for signs/symptoms of serious infections. *Risk D: Consider Therapy Modification*

Desmopressin: Corticosteroids (Systemic) may increase hyponatremic effects of Desmopressin. *Risk X: Avoid*

Deucravacitinib: May increase immunosuppressive effects of Corticosteroids (Systemic). Management: The use of deucravacitinib in combination with potent immunosuppressants is not recommended. Doses equivalent to more than 2 mg/kg or 20 mg/day of prednisone (for persons over 10 kg) administered for 2 or more weeks are considered immunosuppressive. *Risk D: Consider Therapy Modification*

Dinutuximab Beta: Corticosteroids (Systemic) may increase immunosuppressive effects of Dinutuximab Beta. Management: Corticosteroids are not recommended for 2 weeks prior to dinutuximab beta, during therapy and for 1 week after treatment. Doses equivalent to over 2 mg/kg or 20 mg/day of prednisone (persons over 10 kg) for 2 or more weeks are considered immunosuppressive *Risk D: Consider Therapy Modification*

Disulfiram: May increase adverse/toxic effects of Products Containing Ethanol. Management: Do not use disulfiram with dosage forms that contain ethanol. *Risk X: Avoid*

EPHEDrine (Systemic): May decrease serum concentrations of DexAMETHasone (Systemic). *Risk C: Monitor*

Estrogen Derivatives: May increase serum concentrations of Corticosteroids (Systemic). *Risk C: Monitor*

Etrasimod: Corticosteroids (Systemic) may increase immunosuppressive effects of Etrasimod. *Risk C: Monitor*

Filgotinib: Corticosteroids (Systemic) may increase immunosuppressive effects of Filgotinib. Management: Coadministration of filgotinib with systemic corticosteroids at doses equivalent to greater than 2 mg/kg or 20 mg/day of prednisone (for persons over 10 kg) administered for 2 or more weeks is not recommended. *Risk D: Consider Therapy Modification*

Fosamprenavir: DexAMETHasone (Systemic) may decrease serum concentrations of Fosamprenavir. Fosamprenavir may increase serum concentrations of DexAMETHasone (Systemic). *Risk C: Monitor*

Fosaprepitant: May increase serum concentrations of DexAMETHasone (Systemic). Management: Reduce the dexamethasone dose 50% when coadministered with aprepitant. Aprepitant prescribing information incorporates this recommendation into the dose provided for dexamethasone; further reduction is not necessary. *Risk D: Consider Therapy Modification*

Fosnetupitant: May increase serum concentrations of DexAMETHasone (Systemic). Management: Decrease dexamethasone doses to 12 mg on day 1, and if needed based on the emetic potential of the regimen, 8 mg daily on days 2 to 4 of chemotherapy when administered with fosnetupitant. *Risk D: Consider Therapy Modification*

Fosphenytoin: May decrease serum concentrations of DexAMETHasone (Systemic). DexAMETHasone (Systemic) may decrease serum concentrations of Fosphenytoin. DexAMETHasone (Systemic) may increase serum concentrations of Fosphenytoin. Management: Consider dexamethasone dose increases when combined with fosphenytoin and monitor closely for reduced steroid efficacy. Monitor phenytoin levels closely, both increased and decreased phenytoin levels have been reported. *Risk D: Consider Therapy Modification*

Fusidic Acid (Systemic): May increase serum concentrations of CYP3A4 Substrates (High risk with Inhibitors). Management: Consider avoiding this combination if possible. If required, monitor patients closely for increased adverse effects of the CYP3A4 substrate. *Risk D: Consider Therapy Modification*

Gallium Ga 68 Dotatate: Coadministration of Corticosteroids (Systemic) and Gallium Ga 68 Dotatate may alter diagnostic results. *Risk C: Monitor*

Grapefruit Juice: May increase serum concentrations of DexAMETHasone (Systemic). *Risk C: Monitor*

Growth Hormone Analogs: Corticosteroids (Systemic) may decrease therapeutic effects of Growth Hormone Analogs. Growth Hormone Analogs may decrease active metabolite exposure of Corticosteroids (Systemic). *Risk C: Monitor*

Histamine: And Corticosteroids (Systemic) may interact via an unclear mechanism. *Risk C: Monitor*

Hormonal Contraceptives: CYP3A4 Inducers (Weak) may decrease serum concentrations of Hormonal Contraceptives. Management: Advise patients to use an alternative method of contraception or a back-up method during coadministration, and to continue back-up contraception for 28 days after discontinuing a weak CYP3A4 inducer to ensure contraceptive reliability. *Risk D: Consider Therapy Modification*

Hyaluronidase: Corticosteroids (Systemic) may decrease therapeutic effects of Hyaluronidase. Management: Patients receiving corticosteroids (particularly at larger doses) may not experience the desired clinical response to standard doses of hyaluronidase. Larger doses of hyaluronidase may be required. *Risk D: Consider Therapy Modification*

Hydrocortisone (Systemic): CYP3A4 Inducers (Weak) may decrease serum concentrations of Hydrocortisone (Systemic). *Risk C: Monitor*

Imatinib: DexAMETHasone (Systemic) may decrease serum concentrations of Imatinib. Management: Avoid concurrent use of imatinib with dexamethasone when possible. If such a combination must be used, increase imatinib dose by at least 50% and monitor clinical response closely. *Risk D: Consider Therapy Modification*

Immune Checkpoint Inhibitors (Anti-PD-1, -PD-L1, and -CTLA4 Therapies): Corticosteroids (Systemic) may decrease therapeutic effects of Immune Checkpoint Inhibitors (Anti-PD-1, -PD-L1, and -CTLA4 Therapies). Management: Carefully consider the need for corticosteroids, at doses of a prednisone-equivalent of 10 mg or more per day, during the initiation of immune checkpoint inhibitor therapy. Use of corticosteroids to treat immune related adverse events is still recommended *Risk D: Consider Therapy Modification*

Indium 111 Capromab Pendetide: Coadministration of Corticosteroids (Systemic) and Indium 111 Capromab Pendetide may alter diagnostic results. Management: Consider alternatives when possible, as product labeling states the concomitant use of these agents is not recommended. If combined, consider that the diagnostic results obtained with indium 111 capromab pendetide may be altered or unreliable. *Risk D: Consider Therapy Modification*

Inebilizumab: Corticosteroids (Systemic) may increase immunosuppressive effects of Inebilizumab. *Risk C: Monitor*

Influenza Virus Vaccines: Corticosteroids (Systemic) may decrease therapeutic effects of Influenza Virus Vaccines. Management: Administer influenza vaccines at least 2 weeks prior to initiation of systemic corticosteroids at immunosuppressive doses. Influenza vaccines administered less than 14 days prior to or during such therapy should be repeated 3 months after therapy. *Risk D: Consider Therapy Modification*

Isoniazid: Corticosteroids (Systemic) may decrease serum concentrations of Isoniazid. *Risk C: Monitor*

Lapatinib: DexAMETHasone (Systemic) may increase hepatotoxic effects of Lapatinib. DexAMETHasone (Systemic) may decrease serum concentrations of Lapatinib. Management: If therapy overlap cannot be avoided, consider titrating lapatinib gradually from 1,250 mg/day up to 4,500 mg/day (HER2 positive metastatic breast cancer) or 1,500 mg/day up to 5,500 mg/day (hormone receptor/HER2 positive breast cancer) as tolerated. *Risk D: Consider Therapy Modification*

Leflunomide: Corticosteroids (Systemic) may increase immunosuppressive effects of Leflunomide. Management: Increase the frequency of chronic monitoring of platelet, white blood cell count, and hemoglobin or hematocrit to monthly, instead of every 6 to 8 weeks, if leflunomide is coadministered with immunosuppressive agents, such as systemic corticosteroids. *Risk D: Consider Therapy Modification*

Lenacapavir: May increase serum concentrations of DexAMETHasone (Systemic). DexAMETHasone (Systemic) may decrease serum concentrations of Lenacapavir. Management: Consider alternatives to use of dexamethasone with lenacapavir, particularly for long term use. *Risk D: Consider Therapy Modification*

Lenalidomide: DexAMETHasone (Systemic) may increase thrombogenic effects of Lenalidomide. Management: Due to the increased risk of venous and arterial thromboembolism associated with this combination, thromboprophylaxis is recommended, and the choice of regimen should be based on an assessment of the patient's underlying risks. *Risk D: Consider Therapy Modification*

Licorice: May increase serum concentrations of Corticosteroids (Systemic). *Risk C: Monitor*

Loop Diuretics: Corticosteroids (Systemic) may increase hypokalemic effects of Loop Diuretics. *Risk C: Monitor*

Lopinavir: DexAMETHasone (Systemic) may decrease serum concentrations of Lopinavir. Management: Consider alternative corticosteroids for coadministration with lopinavir/ritonavir due to the potential for dexamethasone to decrease lopinavir/ritonavir efficacy and result in the development of resistance. *Risk D: Consider Therapy Modification*

Lutetium Lu 177 Dotatate: Corticosteroids (Systemic) may decrease therapeutic effects of Lutetium Lu 177 Dotatate. Management: Avoid repeated use of high-doses of corticosteroids during treatment with lutetium Lu 177 dotatate. Use of corticosteroids is still permitted for the treatment of neuroendocrine hormonal crisis. The effects of lower corticosteroid doses is unknown. *Risk D: Consider Therapy Modification*

Macimorelin: Coadministration of Corticosteroids (Systemic) and Macimorelin may alter diagnostic results. *Risk X: Avoid*

Methotrimeprazine: May increase CNS depressant effects of Products Containing Ethanol. Management: Avoid products containing alcohol in patients treated with methotrimeprazine. *Risk X: Avoid*

MetyrapONE: Coadministration of Corticosteroids (Systemic) and MetyrapONE may alter diagnostic results. Management: Consider alternatives to the use of the metyrapone test in patients taking systemic corticosteroids. *Risk D: Consider Therapy Modification*

Mifamurtide: Corticosteroids (Systemic) may decrease therapeutic effects of Mifamurtide. *Risk X: Avoid*

MiFEPRIStone: May decrease therapeutic effects of Corticosteroids (Systemic). MiFEPRIStone may increase serum concentrations of Corticosteroids (Systemic). Management: Avoid mifepristone in patients who require long-term corticosteroid treatment of serious illnesses or conditions (eg, for immunosuppression following transplantation). Corticosteroid effects may be reduced by mifepristone treatment. *Risk X: Avoid*

Mumps- Rubella- or Varicella-Containing Live Vaccines: Corticosteroids (Systemic) may decrease therapeutic effects of Mumps- Rubella- or Varicella-Containing Live Vaccines. Corticosteroids (Systemic) may increase adverse/toxic effects of Mumps- Rubella- or Varicella-Containing Live Vaccines. Specifically, the risk of vaccine-associated infection may be increased. *Risk X: Avoid*

Nadofaragene Firadenovec: Corticosteroids (Systemic) may increase adverse/toxic effects of Nadofaragene Firadenovec. Specifically, the risk of disseminated adenovirus infection may be increased. *Risk X: Avoid*

Natalizumab: Corticosteroids (Systemic) may increase immunosuppressive effects of Natalizumab. *Risk X: Avoid*

Netupitant: May increase serum concentrations of DexAMETHasone (Systemic). Management: Decrease dexamethasone doses to 12 mg on day 1, and if needed based on the emetic potential of the regimen, 8 mg daily on days 2 to 4 of chemotherapy when administered with netupitant. *Risk D: Consider Therapy Modification*

Neuromuscular-Blocking Agents (Nondepolarizing): May increase adverse neuromuscular effects of Corticosteroids (Systemic). Increased muscle weakness, possibly progressing to polyneuropathies and myopathies, may occur. Management: If concomitant therapy is required, use the lowest dose for the shortest duration to limit the risk of myopathy or neuropathy. Monitor for new onset or worsening muscle weakness, reduction or loss of deep tendon reflexes, and peripheral sensory decrements *Risk D: Consider Therapy Modification*

Nicorandil: Corticosteroids (Systemic) may increase adverse/toxic effects of Nicorandil. Gastrointestinal perforation has been reported in association with this combination. *Risk C: Monitor*

NiMODipine: CYP3A4 Inducers (Weak) may decrease serum concentrations of NiMODipine. *Risk C: Monitor*

Nirmatrelvir and Ritonavir: May increase serum concentrations of DexAMETHasone (Systemic). *Risk C: Monitor*

Nonsteroidal Anti-Inflammatory Agents (COX-2 Selective): Corticosteroids (Systemic) may increase adverse/toxic effects of Nonsteroidal Anti-Inflammatory Agents (COX-2 Selective). *Risk C: Monitor*

Nonsteroidal Anti-Inflammatory Agents (Nonselective): Corticosteroids (Systemic) may increase adverse/toxic effects of Nonsteroidal Anti-Inflammatory Agents (Nonselective). *Risk C: Monitor*

Nonsteroidal Anti-Inflammatory Agents (Topical): May increase adverse/toxic effects of Corticosteroids (Systemic). Specifically, the risk of gastrointestinal bleeding, ulceration, and perforation may be increased. *Risk C: Monitor*

Ocrelizumab: Corticosteroids (Systemic) may increase immunosuppressive effects of Ocrelizumab. *Risk C: Monitor*

Ofatumumab: Corticosteroids (Systemic) may increase immunosuppressive effects of Ofatumumab. *Risk C: Monitor*

Ozanimod: Corticosteroids (Systemic) may increase immunosuppressive effects of Ozanimod. *Risk C: Monitor*

Phenytoin: May decrease serum concentrations of DexAMETHasone (Systemic). DexAMETHasone (Systemic) may decrease serum concentrations of Phenytoin. Management: Consider dexamethasone dose increases when combined with phenytoin and monitor closely for reduced steroid efficacy. Monitor phenytoin levels closely when combined with dexamethasone, both increased and decreased phenytoin levels have been reported. *Risk D: Consider Therapy Modification*

Pidotimod: Corticosteroids (Systemic) may decrease therapeutic effects of Pidotimod. *Risk C: Monitor*

Pimecrolimus: May increase immunosuppressive effects of Corticosteroids (Systemic). *Risk X: Avoid*

Pneumococcal Vaccines: Corticosteroids (Systemic) may decrease therapeutic effects of Pneumococcal Vaccines. *Risk C: Monitor*

Poliovirus Vaccines (Live/Oral): Corticosteroids (Systemic) may decrease therapeutic effects of Poliovirus Vaccines (Live/Oral). Corticosteroids (Systemic) may increase adverse/toxic effects of Poliovirus Vaccines (Live/Oral). Specifically, the risk of vaccine-associated infection may be increased. *Risk X: Avoid*

Polymethylmethacrylate: Corticosteroids (Systemic) may increase adverse/toxic effects of Polymethylmethacrylate. Specifically, the risk for hypersensitivity or implant clearance may be increased. Management: Use caution when considering use of bovine collagen-containing implants such as the polymethylmethacrylate-based Bellafill brand implant in patients who are receiving immunosuppressants. Consider use of additional skin tests prior to administration. *Risk D: Consider Therapy Modification*

Quinolones: Corticosteroids (Systemic) may increase adverse/toxic effects of Quinolones. Specifically, the risk of tendonitis and tendon rupture may be increased. *Risk C: Monitor*

Rabies Vaccine: Corticosteroids (Systemic) may decrease therapeutic effects of Rabies Vaccine. Management: Complete rabies vaccination at least 2 weeks before initiation of immunosuppressant therapy if possible. If combined, check for rabies antibody titers, and if vaccination is for post exposure prophylaxis, administer a 5th dose of the vaccine. *Risk D: Consider Therapy Modification*

Relacorilant: May decrease therapeutic effects of Corticosteroids (Systemic). Relacorilant may increase serum concentrations of Corticosteroids (Systemic). Management: Using relacorilant with corticosteroids used for lifesaving purposes is contraindicated. Avoid using relacorilant with corticosteroids used for other conditions if possible. If combined, monitor for decreased corticosteroid and relacorilant effectiveness. *Risk D: Consider Therapy Modification*

Rilpivirine: DexAMETHasone (Systemic) may decrease serum concentrations of Rilpivirine. *Risk X: Avoid*

Ritlecitinib: Corticosteroids (Systemic) may increase immunosuppressive effects of Ritlecitinib. Management: Use of ritlecitinib and immunosuppressants is not recommended. Doses equivalent to more than 2 mg/kg or 20 mg/day of prednisone (for persons over 10 kg) administered for 2 or more weeks are considered immunosuppressive. *Risk D: Consider Therapy Modification*

Ritodrine: Corticosteroids (Systemic) may increase adverse/toxic effects of Ritodrine. *Risk C: Monitor*

Ruxolitinib (Topical): Corticosteroids (Systemic) may increase immunosuppressive effects of Ruxolitinib (Topical). Management: The use of topical ruxolitinib and potent immunosuppressants is not recommended. Doses equivalent to more than 2 mg/kg or 20 mg/day of prednisone (for persons over 10 kg) administered for 2 or more weeks are considered immunosuppressive. *Risk D: Consider Therapy Modification*

Salicylates: May increase adverse/toxic effects of Corticosteroids (Systemic). These specifically include gastrointestinal ulceration and bleeding. Corticosteroids (Systemic) may decrease serum concentrations of Salicylates. Withdrawal of corticosteroids may result in salicylate toxicity. *Risk C: Monitor*

Sargramostim: Corticosteroids (Systemic) may increase therapeutic effects of Sargramostim. Specifically, corticosteroids may enhance the myeloproliferative effects of sargramostim. *Risk C: Monitor*

Sebetralstat: CYP3A4 Inducers (Weak) may decrease serum concentrations of Sebetralstat. *Risk C: Monitor*

Secnidazole: Products Containing Ethanol may increase adverse/toxic effects of Secnidazole. *Risk X: Avoid*

Selpercatinib: CYP3A4 Inducers (Weak) may decrease serum concentrations of Selpercatinib. *Risk C: Monitor*

Sibeprenlimab: May increase immunosuppressive effects of Corticosteroids (Systemic). *Risk C: Monitor*

Simeprevir: DexAMETHasone (Systemic) may decrease serum concentrations of Simeprevir. Management: Concomitant use of simeprevir and dexamethasone is not recommended. *Risk D: Consider Therapy Modification*

Sipuleucel-T: Corticosteroids (Systemic) may decrease therapeutic effects of Sipuleucel-T. Management: Consider reducing the dose or discontinuing immunosuppressants, such as systemic corticosteroids, prior to initiating sipuleucel-T therapy. Doses equivalent to more than 2 mg/kg or 20 mg/day of prednisone given for 2 or more weeks are immunosuppressive. *Risk D: Consider Therapy Modification*

Sirolimus (Conventional): CYP3A4 Inducers (Weak) may decrease serum concentrations of Sirolimus (Conventional). *Risk C: Monitor*

Sirolimus (Protein Bound): CYP3A4 Inducers (Weak) may decrease serum concentrations of Sirolimus (Protein Bound). *Risk C: Monitor*

Sitafloxacin: Corticosteroids (Systemic) may increase adverse/toxic effects of Sitafloxacin. Specifically, the risk of tendonitis and tendon rupture may be increased. Management: Consider alternatives to coadministration of corticosteroids and sitafloxacin unless the benefits of coadministration outweigh the risk of tendon disorders. *Risk D: Consider Therapy Modification*

Sodium Benzoate: Corticosteroids (Systemic) may decrease therapeutic effects of Sodium Benzoate. *Risk C: Monitor*

Sphingosine 1-Phosphate (S1P) Receptor Modulators: May increase immunosuppressive effects of Corticosteroids (Systemic). *Risk C: Monitor*

Succinylcholine: Corticosteroids (Systemic) may increase neuromuscular-blocking effects of Succinylcholine. *Risk C: Monitor*

Tacrolimus (Systemic): Corticosteroids (Systemic) may decrease serum concentrations of Tacrolimus (Systemic). Conversely, when discontinuing corticosteroid therapy, tacrolimus concentrations may increase. *Risk C: Monitor*

Tacrolimus (Systemic): CYP3A4 Inducers (Weak) may decrease serum concentrations of Tacrolimus (Systemic). *Risk C: Monitor*

Tacrolimus (Topical): Corticosteroids (Systemic) may increase immunosuppressive effects of Tacrolimus (Topical). *Risk X: Avoid*

Talimogene Laherparepvec: Corticosteroids (Systemic) may increase adverse/toxic effects of Talimogene Laherparepvec. Specifically, the risk of infection from the live, attenuated herpes simplex virus contained in talimogene laherparepvec may be increased. *Risk X: Avoid*

Temsirolimus: DexAMETHasone (Systemic) may decrease active metabolite exposure of Temsirolimus. *Risk C: Monitor*

Tertomotide: Corticosteroids (Systemic) may decrease therapeutic effects of Tertomotide. *Risk X: Avoid*

Thalidomide: DexAMETHasone (Systemic) may increase dermatologic adverse effects of Thalidomide. DexAMETHasone (Systemic) may increase thrombogenic effects of Thalidomide. Management: Consider using venous thromboembolism prophylaxis (eg, low-molecular-weight heparin or warfarin [INR 2.0 to 3.0]) in patients with multiple myeloma receiving both thalidomide and dexamethasone. Monitor for increased dermatologic adverse effects (eg, rash) *Risk D: Consider Therapy Modification*

Thiazide and Thiazide-Like Diuretics: Corticosteroids (Systemic) may increase hypokalemic effects of Thiazide and Thiazide-Like Diuretics. *Risk C: Monitor*

Tofacitinib: Corticosteroids (Systemic) may increase immunosuppressive effects of Tofacitinib. Management: Coadministration of tofacitinib with potent immunosuppressants is not recommended. Doses equivalent to more than 2 mg/kg or 20 mg/day of prednisone (for persons over 10 kg) administered for 2 or more weeks are considered immunosuppressive. *Risk D: Consider Therapy Modification*

Typhoid Vaccine: Corticosteroids (Systemic) may decrease therapeutic effects of Typhoid Vaccine. Corticosteroids (Systemic) may increase adverse/toxic effects of Typhoid Vaccine. Specifically, the risk of vaccine-associated infection may be increased. *Risk X: Avoid*

Ublituximab: Corticosteroids (Systemic) may increase immunosuppressive effects of Ublituximab. *Risk C: Monitor*

Ubrogepant: CYP3A4 Inducers (Weak) may decrease serum concentrations of Ubrogepant. Management: Use an initial ubrogepant dose of 100 mg and second dose (if needed) of 100 mg when used with a weak CYP3A4 inducer. *Risk D: Consider Therapy Modification*

Upadacitinib: Corticosteroids (Systemic) may increase immunosuppressive effects of Upadacitinib. Management: Coadministration of upadacitinib with systemic corticosteroids at doses equivalent to greater than 2 mg/kg or 20 mg/day of prednisone (for persons over 10 kg) administered for 2 or more weeks is not recommended. *Risk D: Consider Therapy Modification*

Urea Cycle Disorder Agents: Corticosteroids (Systemic) may decrease therapeutic effects of Urea Cycle Disorder Agents. More specifically, Corticosteroids (Systemic) may increase protein catabolism and plasma ammonia concentrations, thereby increasing the doses of Urea Cycle Disorder Agents needed to maintain these concentrations in the target range. *Risk C: Monitor*

Vaccines (Live): Corticosteroids (Systemic) may increase adverse/toxic effects of Vaccines (Live). Specifically, the risk of vaccine-associated infection may be increased. Corticosteroids (Systemic) may decrease therapeutic effects of Vaccines (Live). Management: Avoid live vaccines during and for 1 month after therapy with immunosuppressive doses of corticosteroids (equivalent to prednisone > 2 mg/kg or 20 mg/day in persons over 10 kg for at least 2 weeks). Give live vaccines 4 weeks prior to therapy if possible. *Risk D: Consider Therapy Modification*

Vaccines (Non-Live/Inactivated/Non-Replicating): Corticosteroids (Systemic) may decrease therapeutic effects of Vaccines (Non-Live/Inactivated/Non-Replicating). Management: Administer vaccines at least 2 weeks prior to immunosuppressive corticosteroids if possible. If patients are vaccinated less than 14 days prior to or during such therapy, repeat vaccination at least 3 months after therapy if immunocompetence restored. *Risk D: Consider Therapy Modification*

Vitamin K Antagonists: Corticosteroids (Systemic) may increase anticoagulant effects of Vitamin K Antagonists. *Risk C: Monitor*

Voriconazole: DexAMETHasone (Systemic) may decrease serum concentrations of Voriconazole. Voriconazole may increase serum concentrations of DexAMETHasone (Systemic). *Risk C: Monitor*

Yellow Fever Vaccine: Corticosteroids (Systemic) may increase adverse/toxic effects of Yellow Fever Vaccine. Specifically, the risk of vaccine-associated infection may be increased. Corticosteroids (Systemic) may decrease therapeutic effects of Yellow Fever Vaccine. *Risk X: Avoid*

Zoster Vaccine (Live/Attenuated): Corticosteroids (Systemic) may increase adverse/toxic effects of Zoster Vaccine (Live/Attenuated). Specifically, the risk of vaccine-associated infection may be increased. Corticosteroids (Systemic) may decrease therapeutic effects of Zoster Vaccine (Live/Attenuated). *Risk X: Avoid*

Reproductive Considerations

The manufacturer's labeling for use of dexamethasone as part of combination therapy for multiple myeloma recommends pregnancy testing prior to use in patients who could become pregnant. Patients who may become pregnant should use effective contraception during therapy and for at least 1 month after the last dexamethasone dose. Consult individual monographs for additional information related to pregnancy testing and contraception when combination therapy is used for multiple myeloma.

Dexamethasone may alter the motility of and number of spermatozoa.

Pregnancy Considerations

Dexamethasone crosses the placenta (Brownfoot 2013); and is partially metabolized by placental enzymes to an inactive metabolite (Murphy 2007).

Some studies have shown an association between first trimester systemic corticosteroid use and oral clefts or decreased birth weight; however, information is conflicting and may be influenced by maternal dose/indication for use (Lunghi 2010; Park-Wyllie 2000; Pradat 2003). Hypoadrenalism may occur in newborns following maternal use of corticosteroids during pregnancy; monitor.

Dexamethasone is classified as a fluorinated corticosteroid. When systemic corticosteroids are needed in pregnancy for rheumatic disorders, nonfluorinated corticosteroids (eg, prednisone) are preferred. Chronic high doses should be avoided for the treatment of maternal disease (ACR [Sammaritano 2020]).

Use of the overnight dexamethasone 1 mg suppression test for Cushing syndrome is not recommended during pregnancy due to the increased risk of false positives. In addition, dexamethasone is generally avoided for the treatment of pregnant patients with adrenal insufficiency (ES [Nieman 2008]; ESE [Luger 2021]).

Antenatal corticosteroid administration promotes fetal lung maturity and is associated with the reduction of intraventricular hemorrhage, necrotizing enterocolitis, neonatal mortality, and respiratory distress syndrome. A single course of dexamethasone is recommended for patients between 24 0/7 and 33 6/7 weeks' gestation who are at risk of delivering within 7 days. This recommendation includes those with ruptured membranes or multiple gestations. A single course of dexamethasone may be considered for patients beginning at 22 0/7 weeks' gestation who are at risk of delivering within 7 days, in consultation with the family regarding resuscitation. In addition, a single course of dexamethasone may be given to patients between 34 0/7 weeks and 36 6/7 weeks who are at risk of preterm delivery within 7 days and who have not previously received corticosteroids if induction or delivery will proceed ≥ 24 hours and ≤ 7 days; delivery should not be delayed for administration of antenatal corticosteroids. Use of concomitant tocolytics is not currently recommended and administration of late preterm corticosteroids has not been evaluated in patients with intrauterine infection, multiple gestations, pregestational diabetes, or patients who delivered previously by cesarean section at term. Multiple repeat courses are not recommended. However, in patients with pregnancies less than 34 weeks' gestation at risk for delivery within 7 days and who had a course of antenatal corticosteroids >14 days prior, a single repeat course may be considered; use of a repeat course in patients with preterm prelabor rupture of membranes is controversial (ACOG 2016; ACOG 2017; ACOG 2020; ACOG 2022a).

Dexamethasone is used off label in the management of COVID-19. In general, the treatment of COVID-19 during pregnancy is the same as in nonpregnant patients. However, because data for most therapeutic agents in pregnant patients are limited, treatment options should be evaluated as part of a shared decision-making process. Dexamethasone is recommended for hospitalized pregnant patients with COVID-19 if indicated (NIH 2023). In patients who do not require dexamethasone for fetal lung maturity, or in those who have already completed a course of dexamethasone to enhance fetal lung development, treatment recommendations are available using alternative corticosteroids which have more limited placental transfer and may provide less fetal risk. Suggested treatment algorithms are available for pregnant patients with severe or critical COVID-19 requiring corticosteroids (Saad 2020; Teelucksingh 2022). The risk of severe illness from COVID-19 infection is increased in symptomatic pregnant patients compared to nonpregnant patients (ACOG 2023). Information related to the treatment of COVID-19 during pregnancy continues to emerge; refer to current guidelines for the treatment of pregnant patients.

Some products contain alcohol, benzyl alcohol or sodium sulfite; use of preservative-free or alternative formulations in pregnancy is recommended.

Breastfeeding Considerations

Corticosteroids are present in breast milk; information specific to dexamethasone has not been lo-

cated.

The manufacturer notes that when used systemically, maternal use of corticosteroids have the potential to cause adverse events in a breastfeeding infant (eg, growth suppression, interfere with endogenous corticosteroid production).

Single doses of dexamethasone are considered compatible with breastfeeding; information related to prolonged use is not available (WHO 2002). Breastfeeding can continue if dexamethasone is needed for the management of COVID-19 (NIH 2023). However, lactating patients with COVID-19 infection can transmit the virus through respiratory droplets and all precautions should be taken to avoid spreading the virus to the infant (eg, hand hygiene, mask wearing); alternatively, breast milk can be expressed and fed to the infant by someone without confirmed or suspected COVID-19 (ACOG 2023; NIH 2023). Dexamethasone is classified as a fluorinated corticosteroid. When systemic corticosteroids are needed in a lactating patient for rheumatic disorders, low doses of nonfluorinated corticosteroids (eg, prednisone) are preferred (ACR [Sammaritano 2020]).

Due to the potential for serious adverse reactions in the breastfeeding infant, some manufacturers recommend a decision be made to discontinue breastfeeding or to discontinue the drug, considering the importance of treatment to the mother. If there is concern about exposure to the infant, some guidelines recommend waiting 4 hours after the maternal dose of an oral systemic corticosteroid before breastfeeding in order to decrease potential exposure to the breastfed infant (based on a study using prednisolone) (Leachman 2006; Makol 2011; Ost 1985).

The manufacturer's labeling for use of dexamethasone as part of combination therapy for multiple myeloma recommends breastfeeding be discontinued during therapy and for 2 weeks after the last dexamethasone dose.

Dietary Considerations

May be taken with meals to decrease GI upset. May need diet with increased potassium, pyridoxine, vitamin C, vitamin D, folate, calcium, and phosphorus.

Monitoring Parameters

Hb, occult blood loss, BP, serum potassium, blood glucose, creatine kinase (if symptoms of myopathy occur), bone mineral density; intraocular pressure with systemic use >6 weeks; consider routine eye exams with chronic use; weight and height in children; hypothalamic-pituitary-adrenal axis suppression; latent or active amebiasis prior to initiation in patients with recent travel to tropical climates or unexplained diarrhea; reactivation of tuberculosis in patients with latent tuberculosis or tuberculin reactivity.

Oncology patients: Evaluate pregnancy status (in patients who could become pregnant when receiving for multiple myeloma treatment). The American Society of Clinical Oncology hepatitis B virus (HBV) screening and management provisional clinical opinion (ASCO [Hwang 2020]) recommends HBV screening with hepatitis B surface antigen, hepatitis B core antibody, total Ig or IgG, and antibody to hepatitis B surface antigen prior to beginning (or at the beginning of) systemic anticancer therapy; do not delay treatment for screening/results. Detection of chronic or past HBV infection requires a risk assessment to determine antiviral prophylaxis requirements, monitoring, and follow-up.

Mechanism of Action

Dexamethasone is a long-acting corticosteroid with minimal sodium-retaining potential. It decreases inflammation by suppression of neutrophil migration, decreased production of inflammatory mediators, and reversal of increased capillary permeability; suppresses normal immune response. Dexamethasone induces apoptosis in multiple myeloma cells. Dexamethasone's mechanism of antiemetic activity is unknown.

Pharmacokinetics (Adult Data Unless Noted)

Onset of action: IV: Rapid.

Immune thrombocytopenia: Oral: Initial response: 2 to 14 days; Peak response: 4 to 28 days (Neuner 2011).

Duration: IV: Short.

Absorption: Oral: 61% to 86% (Czock 2005).

Metabolism: Hepatic.

Half-life elimination:

Extremely low birth-weight infants with BPD: 9.26 ± 3.34 hours (range: 5.85 to 16.1 hours) (Charles 1993).

Children 4 months to 16 years: 4.34 ± 4.14 hours (range: 2.33 to 9.54 hours) (Richter 1983).

Adults: Oral: 4 ± 0.9 hours (Czock 2005); IV: ~1 to 5 hours (Hochhaus 2001; Miyabo 1981; Rohdewald 1987; Tóth 1999).

Time to peak, serum: Oral: 1 to 2 hours (Czock 2005); IM: ~30 to 120 minutes (Egerman 1997; Hochhaus 2001); IV: 5 to 10 minutes (free dexamethasone) (Miyabo 1981; Rohdewald 1987).

Excretion: Urine (~10%) (Duggan 1975; Miyabo 1981).

Patient Counseling Points

(For additional information see "Dexamethasone (systemic): Patient drug information")

What is this drug used for?

• It is used for many health problems like allergy signs, asthma, adrenal gland problems, blood problems, skin rashes, or swelling problems. This is not a list of all health problems that this drug may be used for. Talk with the doctor.

All drugs may cause side effects. However, many people have no side effects or only have minor side effects. Call your doctor or get medical help if any of these side effects or any other side effects bother you or do not go away:

- Upset stomach or throwing up
- Trouble sleeping
- Restlessness
- Sweating a lot
- Hair thinning
- Headache
- Feeling dizzy, tired, or weak
- Weight gain
- Increased appetite
- Hiccups

WARNING/CAUTION: Even though it may be rare, some people may have very bad and sometimes deadly side effects when taking a drug. Tell your doctor or get medical help right away if you have any of the following signs or symptoms that may be related to a very bad side effect:

All products:

- Infection like fever, chills, flu-like signs, very bad sore throat, ear or sinus pain, cough, more sputum or change in color of sputum, pain with passing urine, mouth sores, or a wound that will not heal
- Low potassium levels like muscle pain or weakness, muscle cramps, or a heartbeat that does not feel normal
- Pancreas problem (pancreatitis) like very bad stomach pain, very bad back pain, or very bad upset stomach or throwing up

- High blood pressure like very bad headache or dizziness, passing out, or change in eyesight
- A weak adrenal gland like a severe upset stomach or throwing up, severe dizziness or passing out, muscle weakness, feeling very tired, mood changes, decreased appetite, or weight loss
- Cushing's syndrome like weight gain in the upper back or belly, moon face, severe headache, or slow healing
- High blood sugar like confusion, feeling sleepy, unusual thirst or hunger, passing urine more often, flushing, fast breathing, or breath that smells like fruit
- Shortness of breath, a big weight gain, or swelling in the arms or legs
- Skin changes (pimples, stretch marks, slow healing, hair growth)
- Purple, red, blue, brown, or black bumps or patches on the skin or in the mouth
- Fast, slow, or abnormal heartbeat
- Chest pain or pressure
- Swelling, warmth, numbness, change of color, or pain in a leg or arm
- Period (menstrual) changes
- Bone or joint pain
- Muscle pain or weakness
- Change in eyesight
- Change in the way you act
- Depression or other mood changes
- Seizures
- A burning, numbness, or tingling feeling that is not normal
- Any unexplained bruising or bleeding
- Severe stomach pain
- Black, tarry, or bloody stools
- Throwing up blood or throw up that looks like coffee grounds
- Signs of an allergic reaction, like rash; hives; itching; red, swollen, blistered, or peeling skin with or without fever; wheezing; tightness in the chest or throat; trouble breathing, swallowing, or talking; unusual hoarseness; or swelling of the mouth, face, lips, tongue, or throat.

Injection:

- Irritation where the shot is given

Note: This is not a comprehensive list of all side effects. Talk to your doctor if you have questions.

Consumer Information Use and Disclaimer: This information should not be used to decide whether or not to take this medicine or any other medicine. Only the healthcare provider has the knowledge and training to decide which medicines are right for a specific patient. This information does not endorse any medicine as safe, effective, or approved for treating any patient or health condition. This is only a limited summary of general information about the medicine's uses from the patient education leaflet and is not intended to be comprehensive. This limited summary does NOT include all information available about the possible uses, directions, warnings, precautions, interactions, adverse effects, or risks that may apply to this medicine. This information is not intended to provide medical advice, diagnosis or treatment and does not replace information you receive from the healthcare provider. For a more detailed summary of information about the risks and benefits of using this medicine, please speak with your healthcare provider and review the entire patient education leaflet.

Brand Names: International

International Brand Names by Country

For country code abbreviations (show table)

- (AE) United Arab Emirates: Apo dexamethasone | Arcodexan | Decadron | Dexa Scheroson | Dexamed | Dexamethasone | Dexona | Mephamesone | Minims dexamethasone;
- (AR) Argentina: Decaaron | Dexalaf | Dexalergin | Dexameral | Dexametasona | Dexametasona dunca | Dexametasona fabra | Dexametasona fecofar | Dexametasona larjan | Dexametasona richet | Dexamfor | Dexatotal | Duo decaaron | Egosona | Fada dexametasona | Lormine | Nexadron | Rupedex;
- (AT) Austria: Dexamethason | Dexamethason hcs | Fortecortin | Infectodexakrupp | Monodex | Oradexon;
- (AU) Australia: Dexamethasone | Dexamethasone s/ph | Dexmethsone;
- (BD) Bangladesh: D-Cort | Decason | Dexa | Dexam | Dexamet | Dexamethason | Dexaminal | Dexan | Dexcor | Dexon | Dexonex | Dextason | Dextor | Gludex | Meradexon | Metadaxan | Odeson | Oradexon | Ordex | Redexon | Rednisone | Sonexa | Steron;
- (BE) Belgium: Aacidexam | Decaaron | Dexamethasone organon | Dexamethasone viatris | Monofree dexamethason | Oradexon;
- (BG) Bulgaria: Decaaron | Dexafree | Dexamethason Krka;
- (BR) Brazil: Acetazona | Bexeton | Cortitop | Decaaron | Decazol | Deflaren | Dexaden | Dexaflan | Dexaglos | Dexametasona | Dexametazona | Dexametonol | Dexamex | Dexan | Dexanil | Dexason | Dexazen | Dexazona | Dexmene | Duo decadron | Koidexa | Metadex | Neodex;
- (CH) Switzerland: Decaaron | Dexacortin | Dexafree | Dexamethason Galepharm | Fortecortin | Mephameson | Millicorten;
- (CI) Côte d'Ivoire: Lodsterod;
- (CL) Chile: Cortyk | Oradexon;
- (CN) China: Dexamethasone | Yi ke tie;
- (CO) Colombia: Alfalyl | Balancort | Decadron | Decadronal | Deltafluorene | Dexalyl | Dexametasona | Duo decadron | Myecort | Neofordex | Oradexon | Tavesona;
- (CR) Costa Rica: Alergodex | Alin depot | Dexagil | Dexanovum | Dexasel;
- (CZ) Czech Republic: Dexamethasone zentiva | Dexamethazon | Fortecortin;
- (DE) Germany: Auxiloson | Cortidexason | Dexa | Dexa Clinit | Dexa loscon mono | Dexa sine | Dexa sine se | Dexaflam | Dexafluid sine | Dexaliquid geriasan | Dexamethason Acis | Dexamethason galen | Dexamethason jenapharm | Dexamethason MP | Dexamethason tad | Dexamonozon | Fortecortin | Infectodexakrupp | Lipotalon | Monodex | Solutio Cordes Dexa N;
- (DO) Dominican Republic: Alin | Arcodexan | Dardex | Decaaron | Dexagesic | Dexalocal | Dexametasona | Dexametasona inmenol | Dexametin | Dexona;
- (EC) Ecuador: Decaaron | Demexam | Dexalaf | Dexamark | Dexametasona | Dexametazona | Duo decaaron;
- (EE) Estonia: Dexamed | Dexametason | Dexamethason | Dexamethason galen | Dexamethason Krka | Dexamethasonum | Nodexon;
- (EG) Egypt: Decaaron | Deltasone | Dexamethasone | Dexazone | Fortecortin | Methabiogen | Oradexon | Orazone | Orchamethasone;
- (ES) Spain: Dexafree | Dexametasona abdrug | Dexametasona krka | Fortecortin | Fridex;
- (ET) Ethiopia: Prednisolon f | Xsone;
- (FI) Finland: Decadron | Dexametason | Dexametason Abcur | Dexamethason star | Dexamethasone orifarm | Kaarmepakkaus | Monopex | Neofordex;
- (FR) France: Decadron | Dectancyl | Dexamethasone PCH | Dexliq | Neodex | Neofordex;
- (GB) United Kingdom: Decaaron | Dexacortisyl | Dexafree | Dexamethasone | Dexamethasone phosphate | Dexsol | Dropodex | Martapan | Neofordex | Oradexon;
- (GR) Greece: Amelinor | Axium | Axium opto | Decaaron | Decaaron/Visufarma | Deksametazon Krka | Dexa dose | Dexafree | Dexaleau | Dexamethasone/gap | Dexamethasone/Rosemont | Dexametazone | Dexeta | Draibe | Neofordex | Oradexon | Perazone | Suomi | Suomicon | Thilodexine;

- (GT) Guatemala: Dexametasona | Dexametasona flamingo;
- (HK) Hong Kong: Apo dexamethasone | Cp dexta | Daxilone | Deca | Decadron | Dexamed | Dexasone | Dexatab | Dexmetha | Dexmethasone | Dexmethsone | Dexone | Euromethasone | Kovin tong | Scandexon | Zotex;
- (HR) Croatia: Deksametazon Krka | Deksametazon pliva | Deksametazon zentiva | Dexamethason Krka;
- (HU) Hungary: Dexacort forte | Dexamethasone Krka | Oradexon;
- (ID) Indonesia: Adrekon | Benodex | Carbidu | Cellacort | Cetadexon | Corsona | Cortidex | Danasone | Decilone | Dellamethasone | Deltafluorene | Dexa-m | Dexacort | Dexamethasone | Dexanel | Dexicorta | Dexon | Diometa | Ermethasone | Etason | Faridexon | Fexadron | Fortecortin | Grathazon | Hopamethasone | Ifidex | Indexon | Kaldexon | Kalmethasone | Lanadexon | Licodexon | Mecoxon | Mediamethasone | Megadex | Megasonum | Molacort | Nufadex m | Omedexon | Oradexon | Pidexon | Prodexon | Pycameth | Pyradexon | Scandexon | Selexon | Syndexa | Zecaxon;
- (IE) Ireland: Decadron | Dexafree | Dexsol | Dropodex;
- (IL) Israel: Dexacort | Dexamethazone;
- (IN) India: Adesone | Auradex | Biodexone | Cortidex | Deca forte | Decdak st | Decdan | Deexa | Deksa | Demson | Dexacan | Dexacip | Dexacron | Dexagee | Dexamag | Dexamethasone | Dexamine | Dexane | Dexasone | Dexinga | Dexona | Dicorsone | Idizone | Mycidex | Randexa | Stedex | Wymesone | Zuvidexa | Zydexa;
- (IQ) Iraq: Awzaron | Dexamethasone aljazeera | Dixsoon | Furadex | Ipisone | Kindixone;
- (IT) Italy: Decadron | Desadoc | Eta cortilen | Oftacortal | Soldesam | Tamesad | Usoldec | Visu-cortex;
- (JO) Jordan: Decadron | Dexamed;
- (JP) Japan: Carulon | Corson | Daidoron | Decadron | Dectan chugai | Deron s | Dethamedin | Dexa | Dexa ferosone | Dexa Scheroson | Dexamethasone asahi kasei | Dexamethasone fuso | Dexamethasone isei | Dexamethasone kowa | Dexamethasone merck hoei | Dexamethasone mitsubishi | Dexamethasone nipro | Dexamethasone organon | Dexamethasone s.s. seiyaku | Dexamethasone sawai | Dexamethasone showa | Dexamethasone taisho | Dexamethasone taiyo | Dexamethasone tobishi | Dexamethasone towa | Dexasone | Donray | Elixirmethasone | Lenadex | Limethason | Limethason gc | Loverine | Metasolon | Mitasone | Orgadrone | Santeson | Sawasone | Solcort;
- (KE) Kenya: Dexafil | Dexasone | Omnidex;
- (KR) Korea, Republic of: Deltafluorene | Dexa | Dexa Scheroson | Dexacotisol | Dexahigh | Dexamethasone bkx | Dexan | Dexasolone | Dexasone | Dextason | Limethason | Megadexa | Oradexon | Samnam dexamethasone | Sindexa | Sinil dexamethasone | Yuhan dexamethasone;
- (KW) Kuwait: Decadron | Dexafree | Dexsol;
- (LB) Lebanon: Decadron | Dexafree | Dexamed | Oradexon;
- (LT) Lithuania: Dexa Edo | Dexa sine se | Dexafree | Dexamed | Dexamethason | Dexamethason Krka | Dexason | Dexona | Monodex | Nodexon;
- (LU) Luxembourg: Aacidexam | Fortecortin | Monofree Dexamethason | Oradexon;
- (LV) Latvia: Dectancyl | Dexamethason | Dexason | Maxi dex | Nodexon | Orlixon;
- (MA) Morocco: Decadron | Dexafree | Oradexon;
- (MX) Mexico: Adrecort | Alin | Alin depot | Alleffective | Benimex | Decadron | Decadronal | Decorex | Dexagrin | Dexametasona | Dibasona | Etacortilen | Metax | Migradexan | Neptonax | Oradexon | Wiserdex;
- (MY) Malaysia: Cortidax | Decadron | Decan | Detasone | Dexa | Dexalone | Dexamethasone | Dexasone | Dexin | Dextrasone | Erladexone | Mexasone | Minims dexamethasone | Roximeth | Vextasone;
- (NG) Nigeria: Alapharm dexamethasone | Bhudex | Bleor dexamethasone | Climax dexamethasone | Delisa | Dexacee | Dexacure | Global young dexamethasone | Greenlife dexamethasone | Hydexa | Jamdexa | Kris dexamethasone | Lifeline dexamethasone | Liricon dexamethasone | Macdexa | Marley dexamethasone | Medexone | Nci odia | Nkoyo dexamethasone | P one dexamethasone | Rebok dexamethasone | Rich dexa | Ricnos dexamethasone | Sivodexa | Surelife dexamethasone | Talidexa | Timec dexamethasone | Updexa | Xasten | Yellow dexamethasone;

- (NL) Netherlands: Decaaron | Dexamethason | Dexamethason Apotex | Dexamethason CF | Dexamethason PCH | Dexamethason pharmamedic | Monofree dexamethason | Oradexon;
- (NO) Norway: Decadron | Deksametason aurora medical | Dexa sine | Dexacortin | Dexagalen | Dexamethason | Dexamethason Abcur | Dexamethason jenapharm | Dexamethasone Krka | Dexaton | Fortecortin | Monopex | Neofordex;
- (NZ) New Zealand: Decadron | Dexmethsone;
- (PA) Panama: Alergodex | Alin | Alin depot | Dexagil | Dexametasona;
- (PE) Peru: Banter | Branxona | Cortisciens | Decaaron | Decorpred | Decorten | Desazona | Dexa find | Dexabiotech | Dexacler | Dexacort | Dexafar | Dexaflam elixir | Dexalab | Dexaler | Dexaluxor | Dexam | Dexamark | Dexamax e | Dexamedix | Dexametan | Dexametasona | Dexamizan | Dexaplus | Dexasona | Dexators | Dexavet | Dexpharm | Dxa four | Lipdex | Medasonil | Medicort | Medicort nf | Oradexon | Quillacort | Sedexxa | Terbometasona | Tetracort;
- (PH) Philippines: Decaaron | Decilone | Dexacort | Dexamethason | Dexamethasone Gamot | Dexat | Dexone | Drenex | Impodex | Inflam | Oradexon | Sterasone | Sydencort | Vexamet;
- (PK) Pakistan: Aldron | Decadron | Decasone | Dexadrin | Dexadron | Dexamethasone | Dexason | Dexawal | Duo aecadron | Fortecortin | Irzamethasone | Kanadex | Maxidex | Oradexon | Shirodex;
- (PL) Poland: Demezoon | Dexafree | Dexamecortin | Dexamethason | Dexamethasone Krka | Dexason | Pabi-dexamethason;
- (PR) Puerto Rico: Active injection kit d | Ak-Dex | Decadron | Dexabliss | Dexamethasone | Dexamethasone Intensol | Dexpak jr | Dmt suik | Dxevo | Hemaay | Locort | Mas care pak | Readysharp aexamethasone | Taperdex | Topiaex | Zcort | Zema-Pak | Zodex | Zonacort;
- (PT) Portugal: Decaaron | Dexafree | Dexametasona kabi | Dexametasona krka | Dexamethasone | Fortecortin;
- (PY) Paraguay: Cipro cort a nf | Decaaron | Dexametasona celsius | Dexametasona dallas | Dexametasona heisecke | Metadin d nf | Monocort | Zicacort;
- (QA) Qatar: Apidexol | Decaaron | Dekort | Deksamet | Dexamed Phosphate | Dexamethasone Intensol | Dexsol | Gadexon | Tikatrom;
- (RO) Romania: Dexamed | Dexametazona krka | Dexamethason | Perazone;
- (RU) Russian Federation: Dexamed | Dexamethason | Dexamethasone Krka | Dexason | Dexona | Megadexan;
- (SA) Saudi Arabia: Apo aexamethasone | Decaaron | Demson | Dexamed | Millicorten | Oradexon | Pms-aexamethasone;
- (SE) Sweden: Baxil | Decaaron | Dexacortal | Dexafree | Dexametason 2care4 | Dexametason Abcur | Dexametason ebb | Dexametason rosemont | Dexamethasone abacus medicine | Dexamethasone Krka | Dexamethasone orifarm | Dexamethasone paranova | Dexamethasone zentiva | Neofordex;
- (SG) Singapore: Decan | Dexalone | Dexamed | Dexasone | Dexmetha | Dextrasone | Erladexone | Mexasone | Minims aexamethasone | SP Cordexa;
- (SI) Slovenia: Dexamethason | Dexamethason galen | Dexamethason jenapharm | Dexamono | Fortecortin;
- (SK) Slovakia: Dexametazon krka | Dexamethazon;
- (SL) Sierra Leone: Dxone;
- (SR) Suriname: Demasone | Dexamethason | Dexamethasonum;
- (TH) Thailand: B Dexzol | B.Dexzol W | Decaaron | Dexa | Dexa k | Dexa PS | Dexa-o | Dexa-p | Dexa-Win | Dexa-y | Dexamethasone | Dexamethasone T Man | Dexamo | Dexano | Dexapro | Dexasin | Dexasone | Dexatab | Dexazone | Dexone | Dexstar | Dexthasol | Dexton | Duo aecadron | Limethason | Lodexa | Medexasone | Methasone | Oradexon | Shor's Dexasone | Uto aexamethasone | Xasone;
- (TN) Tunisia: Decadron | Dectancyl | Dexamedis Fort | Oradexon | Pms-aexamethasone;
- (TR) Turkey: Dekort | Deksalon | Dexa sine | Kordexa | Onadron;
- (TW) Taiwan: Ancom | Bucokon | Canalon | Deca | Decadlin | Decadolone | Decaaron | Decalin | Decalon | Decan | Decans | Decaron | Decarone | Decason | Decon | Decone | Decoron | Decoton | Dekesu | Demeson | Deson | Dexacortin | Dexadon | Dexadrol | Dexamethasone | Dexan | Dexaron | Dexasolen | Dexasolon | Dexason | Dexasone | Dexazone | Dexmesone | Dexon | Dexsone | Dicosheun | Dison | Dorison | Kovintong | Limeson | Medesone | Methasone | Mexasone | Neosolon | Oradexon | Rocolone | Shuayan | Sundron | Teanlang | Tekuron | Ucalon | Unisone;

- (UA) Ukraine: Dexafree | Dexamethason | Dexason | Dexona;
- (UG) Uganda: Agomethasone | Axasone | Dexariv | Dexona | Infudexa | Perazone | Prodexa t | Xsone;
- (UY) Uruguay: Corodex | Dexametasona | Dexametasona Efa | Dinedrox;
- (VE) Venezuela, Bolivarian Republic of: Decaaron | Decalona | Decobel | Decorex | Dekort | Dexafar | Dexamed | Dexametasona | Dexason | Flesmodex;
- (VN) Viet Nam: Dextazyne | Tiphadeltacil;
- (ZA) South Africa: Decaaron | Minims Dexamethasone sodium phosphate | Neofordex | Oradexon | Perazone;
- (ZM) Zambia: Demson;
- (ZW) Zimbabwe: Dexone

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